NIHR | National Institute for Health and Care Research

Reference number	NIHR206718
Programme	Research for Patient Benefit
Call	RfPB Competition 51 Stage 2
Lead Applicant	Professor Peter Butler
Host organisation	Royal Free London NHS Foundation Trust
Research Title	PlatEleT-rich plasma and Autologous lipotransfer to treat vulval fibrosis in Lichen Sclerosus – the PETALS multicentre randomised controlled feasibility trial
Start Date	01/09/2024
Grant Duration (Months)	34
End Date	
Research Costs	£248,575.00
Research Type	Primary Research

Pre-submission

Table Of Contents

1. The Research Team
2. History of application
3. Scientific abstract
4. Plain English Summary
5. Changes from first stage
6. Detailed Research plan
7. Patient & Public Involvement
8. Detailed Budget
9. Management & Governance
10. Uploads
11. Administrative contact details
12. Research & Development office contact
13. Acknowledgement, review and submit
14. CV - Lead Applicant(s)
15. CV - Co-applicants

Pre-submission

1. The Research Team

Specify your (lead applicant) role in the research
As a lead applicant with >20 years of consultant experience in managing fibrotic conditions, including vulval lichen sclerosus, scleroderma and radiation-induced fibrosis and an extensive clinical trial track record, I will have the overall responsibility for the success of this trial and its completion within the time and budget constraints. I will oversee the ethics application to ensure that it has been granted before the trial funding commences. I will encourage and support patient recruitment. My role will also include promoting the trial at national and international conferences. Additionally, I will supervise the project management group.

% FTE Commitment: This refers to the percentage of your time that you will commit to this project. If you are funded as part of other NIHR projects that will be running concurrently, your time must not exceed 100% overall.
10

Co-applicant role and % FTE commitment
Specify the role and percent FTE commitment of each co-applicant.

Full Name	Miss Aurora Almadori	Institution	Royal Free and University College Medical School

Please indicate if this co-applicant is a member of the public
No

Specify role in research
Co-applicant, involved in trial design, coordination and management.

% FTE Commitment: This refers to the percentage of your time that you will commit to this project.
4

Full Name	Professor Norman Williams	Institution	University College London

Please indicate if this co-applicant is a member of the public
No

Specify role in research
Co-applicant involved in trial design, methodological input, conduct and management of the trial and data analysis.

Pre-submission

% FTE Commitment: This refers to the percentage of your time that you will commit to this project.
5

Full Name	Dr Hakim-Moulay Dehbi	**Institution**	University College London

Please indicate if this co-applicant is a member of the public
No

Specify role in research
Co-applicant involved in trial design, methodological input and statistical analysis.

% FTE Commitment: This refers to the percentage of your time that you will commit to this project.
5

Full Name	Miss Monica Panca	**Institution**	University College London

Please indicate if this co-applicant is a member of the public
No

Specify role in research
Co-applicant, involved in trial design, health economics input and analysis

% FTE Commitment: This refers to the percentage of your time that you will commit to this project.
5

Full Name	Dr Esther Hansen	**Institution**	Royal Free Hospital

Please indicate if this co-applicant is a member of the public
No

Specify role in research
Co-applicant, psychological counselling and advice, PPI oversight

Pre-submission

% FTE Commitment: This refers to the percentage of your time that you will commit to this project.
1

Full Name	Dr Rosalind Simpson	**Institution**	University of Nottingham

Please indicate if this co-applicant is a member of the public
No

Specify role in research
Co-applicant with Dermatology and vulval disease expertise, recruitment lead for the second recruitment centre.

% FTE Commitment: This refers to the percentage of your time that you will commit to this project.
2.5

Full Name	Dr Victoria Swale	**Institution**	Royal Free London NHS Foundation Trust

Please indicate if this co-applicant is a member of the public
No

Specify role in research
Co-applicant with Dermatology and vulval disease expertise at Royal Free Hospital.

% FTE Commitment: This refers to the percentage of your time that you will commit to this project.
1

Full Name	Mrs Maria Tether-Jenkins	**Institution**	PPI representative

Please indicate if this co-applicant is a member of the public
Yes

Please include a clear description of their role and the reasons why a public co-applicant is joining the team. Co-applicants who are patients, service users or carers are not obliged to complete a standard CV but are required to provide a summary of any knowledge, skills and experience relevant to their role in the

Pre-submission

application.

Ms Maria Tether-Jenkins, our patient co-applicant, has been actively involved in the design of this trial and has had considerable input into the proposed patient and public involvement. She has 40 years of lived experience of dealing with VLS daily having had it since early childhood but not being diagnosed until the age of 30.

She is passionate about finding new treatments for this disease and understands the importance of increasing the awareness of VLS signs and symptoms to increase the number of women diagnosed early.

Maria has undergone three autologous fat transfer operations under the care of Professor Butler and has found this treatment life-changing. Thanks to surgical management, long-term steroid treatment and lifestyle changes she can live a normal and pain-free life.

She has been working alongside Professor Butler as a Patient advisor for women diagnosed with VLS at Royal Free Hospital. She is an active member of Facebook support groups (11,000 members) where she shares her experience with a wider VLS community daily.

She has developed research and leadership skills through running her own health food supplement company.

Maria will be actively involved during all stages of this study and will be invited to attend the regular Trial Management Group meetings. She will be a PPI lead and will work alongside our consultant clinical psychologist co-applicant and an academic clinical fellow who will help her at every stage. Maria will help us decide on the best way to disseminate the results of the study and raise awareness of this condition and the proposed treatments.

Specify role in research

Public co-applicant, PPI lead, patient representative, admin of a Facebook Vulval Lichen Sclerosus patient support group and forum.

% FTE
Commitment: This refers to the percentage of your time that you will commit to this project.

5

Full Name	Dr Sara Jasionowska	**Institution**	Royal Free Hospital

Please indicate if this co-applicant is a member of the public

No

Specify role in research

Co-applicant assisting the CTU in site set-up, coordination of the trial and its conduct, management and analysis, clinical PPI support and oversight, and outcome assessment.

% FTE
Commitment: This refers to the percentage of your time that you will commit to this project.

10

Full Name	Dr Benjamin Langridge	**Institution**	Royal Free Hospital

Pre-submission

Please indicate if this co-applicant is a member of the public
No

Specify role in research
Co-applicant, involved in assisting the CTU in site set-up, coordination of the trial and its conduct, management and analysis and outcome assessment.

% FTE Commitment: This refers to the percentage of your time that you will commit to this project.
5

2. History of application

Has this application been previously submitted to this or any other funding body?
No

3. Scientific abstract

Scientific abstract

Background

Vulval Lichen Sclerosus (VLS) is a chronic inflammatory skin disease that affects the vulva and anogenital region (1). Its prevalence is up to 3% (2,3), with peak onset in prepubertal girls and postmenopausal women. Symptoms include itching and pain, with flat, porcelain-white lesions. Scarring and loss of vulval architecture can develop in up to 79% of cases (4). Dyspareunia caused by fourchette fissuring and erosions and psychological impact further complicate the condition and can have a significant impact on quality of life including intimacy and physical activity. First-line treatment includes topical corticosteroids (TCS) which are effective during the acute and maintenance phases but can be insufficient in managing scarring and structural changes. Other treatments have limited efficacy evidence. New surgical options including autologous fat transfer (AFG) and platelet-rich plasma (PRP) have shown the potential to improve VLS symptoms and VLS-related scarring and loss of architecture but lack robust prospective clinical evidence. This study aims to evaluate the feasibility of running a definitive trial investigating these treatments in VLS.

Aims

The primary feasibility outcome will be the proportion of eligible patients willing to be randomised. We will also assess the adherence to the trial protocol, the feasibility of setting up other treatment centres across the UK, running a multicentre trial and the acceptability of the treatments to patients and clinicians. We will collect preliminary clinical efficacy and safety data to identify the most suitable outcome measures and calculate the sample size required for an adequately powered future multicentre trial.

Methods

We propose a single-blind (blinded assessors) randomised controlled feasibility study with a 1:1:1 randomisation into three treatment arms: TCS only (Standard of Care – SoC), AFG + SoC and PRP + SoC. Patients in two surgical arms will undergo a procedure under general anaesthesia where AFG and PRP will be collected and either AFG or PRP will be injected into the vulval area. Patients will be blinded to the injection they receive. A pragmatic approach of allowing patients to continue a potent TCS which they are known to tolerate will be used. Patients with poorly controlled disease will be reviewed by a Consultant Dermatologist. We plan to recruit up to 75 patients. The inclusion criteria are adults (>18 years), female sex, patients with expert-diagnosed clinical and/or histological diagnosis of VLS, able to give informed consent and communicate in English, and VLS-related structural changes. Patients will be followed up over the phone during the immediate postoperative period. Follow-up visits will take place at Royal Free Hospital at 3 and 6 months.

4. Plain English Summary

Pre-submission

Plain English summary

Background

Vulval lichen sclerosus (VLS) is an itchy condition affecting the skin around the entrance to the vagina (vulva). In severe cases, it causes scarring and fusion of parts of the vulva, leading to painful skin tears, difficulty in having sex and going to the toilet. VLS affects women of any age but is most diagnosed before puberty and after menopause. Treatment includes lifelong use of strong corticosteroid creams, which are effective for most women. For most patients the condition is controlled by this treatment but some experience ongoing symptoms, scarring and skin thickening. Recent studies have introduced two potential surgical treatments: fat grafting and platelet-rich plasma which may improve VLS symptoms and reduce scarring. Fat grafting involves collecting fat from the patient's own body and injecting it into the vulva. Platelet-rich plasma is obtained from the patient's blood, prepared in a special way, and then injected into the vulva.

Aims

We want to carry out a feasibility (pilot) study in which patients will either undergo one of these new treatments or use corticosteroid cream only to check if these treatments are acceptable and how to best measure their effectiveness. We also want to know if it is possible to perform a larger trial in multiple hospitals in the future to find out if these treatments work.

Design and methods

This study will involve up to 75 women with VLS. They will be allocated randomly and equally into three groups: fat grafting, platelet-rich plasma and corticosteroid cream only. Patients in the surgical groups will undergo one surgery and will continue using the corticosteroid cream. Participants will complete questionnaires about their symptoms, quality of life and sexual function. We will assess improvements by analysing photographs of the vulva and measuring the thickness of the vulval skin using specialised equipment. Patients will be followed up for six months.

Patient and public involvement

Our patient representative, a woman with VLS, has contributed to the study design. We also conducted an online survey involving 809 women with VLS to shape the study. The patient representative will remain on the trial committee and participate in decision-making throughout the study. We established a patient advisory group of women with VLS to gather their ongoing opinions and ensure our research aligns with patients' priorities.

Dissemination

The results of this study will be used to determine the design of a large trial which will allow us to check if the new treatments work and whether they can be offered to patients across the UK. We will create an online platform where the results will be disseminated. The results will be shared through patient support groups, social media, medical conferences and academic journals.

5. Changes from first stage

Pre-submission

NIHR | National Institute for Health and Care Research

Professor Peter Butler
By Email

NIHRCC
Grange House
15 Church Street
Twickenham
TW1 3NL

Tel: 020 8843 8000
Email: rfpb@nihr.ac.uk
www.nihr.ac.uk

Wednesday 18 October 2023

Dear Professor Butler,

Research for Patient Benefit (RfPB) Programme: NIHR206718 – PlatEleT-rich plasma and Autologous lipotransfer to treat vulvar fibrosis in Lichen Sclerosus – the PETALS multicentre randomised controlled feasibility trial

Following the Stage 1 review of applications submitted to RfPB Competition 51, I am pleased to inform you that your application has been invited to progress to Stage 2 of the application process.

Further to our previous correspondence, I include your feedback from the Committee below. You are advised to address these feedback points in Stage 2 of the application process.

The Committee made the following comments:

- Please provide information on Medicines and Healthcare products Regulatory Agency (MHRA) correspondence in relation to the proposed therapies: PlatEleT-rich plasma and Autologous lipotransfer.

- Concerns were raised regarding some aspects of the research plan and methodology, and the applicants were invited to address them in the Stage 2 application. Specifically, the applicants were asked to:
    - Provide more details on recruitment and discuss the feasibility of meeting recruitment targets through only one site.
    - Discuss what consideration had been given to the impact of travel between London and Nottingham on participants and recruitment.
    - Justify the inclusion of efficacy outcomes in a feasibility study.
    - Consider the inclusions of a sham procedure in the control arm with the aim of strengthening the evidence obtained from a definitive trial.
    - Provide justification how adherence to protocol and acceptability of treatment to clinicians can be assessed with RFH as the only study centre and the only centre in the country already providing autologous fat transfer.

- The Committee recommended further development of the patient and public involvement (PPI) component. In particular, additional details about how the PPI representatives would be involved in the study and how their views would be taken into account were requested. The team were also requested to justify the inclusion of the PPI member at only 1% FTE.

- The Committee requested more information on the prevalence of vulvar fibrosis in Lichen Sclerosus (VLS) in all populations and how many patients seek treatment through primary and secondary care.

- The Committee sought clarity on the progression criteria to the full trial, and requested the team consider the use of the “traffic light” system.

- The Committee requested that the plain English summary (PES) be rewritten to remove the excess jargon and acronyms and to also include an explanation of vulvar fibrosis in Lichen Sclerosus. Please note that the word limit for a PES is 450 words.

The attached outcomes table contains the anonymised mean scores and ranking from the Competition 51 Stage 1 Regional Assessment Committee meeting where your application was assessed. It is intended to aid you in contextualising your feedback from the Committee.

Your anonymised code is: LDN_0923_0004

The Stage 2 application form is now available to complete on the Research Management System and the deadline for submission is **1pm** on **Wednesday 22 November 2023**.

Please be aware that applications should be accompanied with the ‘Funder Export’ from the online SoECAT, obtainable via the NIHR Central Portfolio Management System (CPMS). Further guidance is available via the RMS application.

Further information on the RfPB Programme, including guidance notes and frequently asked questions, can be found on the RfPB website. The RfPB Programme also encourages applicants to share the Committee’s feedback and speak with the NIHR Research Support Service (RSS) for further advice.

I wish you all the best for Stage 2 of the Competition.

Yours sincerely,

Mr Richard Foulsham
Programme Manager

Changes from Stage 1

1. Please provide information on Medicines and Healthcare products Regulatory Agency (MHRA) correspondence in relation to the proposed therapies: PlatEleT-rich plasma and Autologous lipotransfer.

Autologous fat grafting (AFG); Has been used in clinical practice by Professor Peter Butler since 2001. Professor Peter Butler met with the Medicines and Healthcare Products Regulatory Agency (MHRA) in 2017 and after discussion with the MHRA was informed that this procedure is considered under the regulations as resizing and not a cell therapy and therefore does not require MHRA approval as it has no manipulation other than centrifugation as per standardised practice in the operating room before injection. Minutes from the meeting have been provided as an attachment (Appendix_MHRA correspondence) This meeting occurred before its use in a clinical trial.

Platelet Rich Plasma; PRP is used clinically in many conditions including vulval lichen sclerosus (VLS). We met with David R Jones FRSB FBTS Expert Pharmaco-Toxicologist who communicated with the MHRA on our behalf regarding this feasibility trial. We were informed that if the PRP used was not altered with calcium chloride or other additive it is not considered an investigational medicinal product and therefore does not need MHRA approval. Furthermore, this is a feasibility trial and is not a clinical trial and therefore does not need MHRA approval. The results of this feasibility trial will inform the design of subsequent a prospective randomised multicentre clinical trial. Attached is a copy of email correspondence with the MHRA confirming this approach (Appendix_MHRA correspondence).

2. Provide more details on recruitment and discuss the feasibility of meeting recruitment targets through only one site.

Potential participants will be recruited from two sites: Royal Free Hospital and Nottingham University Hospitals.

Approximately 500 patients are seen at the Vulval clinic at Royal Free Hospital each year, which includes 380 new patient referrals with VLS. Additionally, there are 200 VLS patients seen at Nottingham University Hospital each year. We also have identified two additional patient referral centres: the Department of Dermatology, Southampton (under Dr Helen Lowery and Dr Alice Plant) and the Department of Dermatology, Norwich (under Dr Karen Gibbon). There are 350 new VLS patients seen at Southampton each year.

Minimum target recruitment of 3 patients per month has been estimated as outlined in the traffic light system (Table 4). This has been set as a minimum number for the outcome of this feasibility trial with a number of mitigations to achieve and exceed this minimum recruitment. We plan to recruit a maximum of 25 patients per group which would result in a total sample size of 75.

We are going to review the recruitment rate during regular Trial Management Group meetings and will consider identifying additional referral sites if the recruitment rate falls within the red or amber criteria outlined in Table 4.

We are also going to increase awareness of the trial through the VLS online patient support groups, national dermatology, gynaecology and plastic surgery conferences and collaboration with agencies like the British Society for the Study of Vulval Disease (BSSVD) (Please see the Appendix_BSSVD Support Letter) and Wellbeing of Women (WoW) charity to increase feasibility trial recruitment rates.

3. Discuss what consideration had been given to the impact of travel between London and Nottingham on participants and recruitment.

Currently, patients are referred to Professor Butler from across the UK for lipotransfer treatment.

For this study, we will offer to reimburse the cost of travel for all eligible patients. We have increased the provision for the travel cost from Nottingham as well as any other recruitment sites. We estimated this based on an average transport cost from Nottingham. This is detailed in the SoECAT form.

This will reduce the impact of travel from Nottingham, Norwich, Southampton and other locations on patients and therefore improve recruitment and retention.

The impact of the distance travelled to the treatment centre on patient recruitment and retention will be one of the feasibility outcomes of this study.

We will assess the impact of travel on patients and recruitment through questionnaires administered to all eligible patients regardless of whether they agreed to participate or not. We will therefore be able to monitor if travel is an issue and mitigate this risk if possible but also use this data to support the design of a future clinical trial.

Pre-submission

4. Justify the inclusion of efficacy outcomes in a feasibility study.

The randomised prospective data on the proposed treatments is very poor in the literature. The utility of the proposed outcome measures is not adequately assessed in this cohort of patients in the literature. Their inclusion will help us determine whether these measures are appropriately sensitive to change and should be or can be included in the future trial.

We will assess the acceptability of the proposed outcome assessments to patients and clinicians and decide which assessment tools should be used in the definitive trial.

The inclusion of efficacy outcomes will also allow us to calculate a sample size for the adequately powered definitive trial and establish the appropriate follow-up schedule and duration.

5. Consider the inclusions of a sham procedure in the control arm with the aim of strengthening the evidence obtained from a definitive trial.

We have previously considered including a sham procedure (saline injections under general anaesthesia) in addition to topical corticosteroids in the control group of this study.

We have presented the study protocol to members of the British Society for the Study of Vulval Disease (BSSVD) and discussed the inclusion of a sham procedure. The majority recognised the advantage of the trial design which includes a sham procedure, but some raised ethical concerns about performing a placebo procedure under general anaesthesia. Others were concerned about the possibility of exacerbating VLS symptoms by injection of saline into the disease-affected area. Therefore, we feel this is something we need to explore in this feasibility study.

We have also discussed the inclusion of a sham procedure with our patient groups during our PPI work. The sham procedure would involve a patient undergoing a procedure under general anaesthesia, harvesting of abdominal fat with a standard liposuction cannula and 20mls of blood from the antecubital veins which is the source of PRP all of which would be discarded. All patients who attended the initial focus group (n=18) said that they would not take part in this study if a sham procedure was included. We posed the same question to a larger group of 809 patients through an online questionnaire. Seventy-three percent of respondents said that they would be willing to take part in the study if a sham procedure was used. The most common reasons for refusing to take part if a sham procedure was used included pain and fear of worsening symptoms.

Because of the mixed opinions from both patients and clinicians on the inclusion of a sham procedure, we need to examine the acceptability or lack thereof. This may impact not only our ability to recruit and retain patients in a future clinical trial but also our ability to achieve the largest potential clinical referral to this potential future trial. We are concerned that including a sham procedure in this study might affect the recruitment rate before the patients are referred to our centre and we might not be able to assess if it is a barrier to recruitment. We will add the acceptability of including a sham procedure in the definitive trial as one of the outcomes of this feasibility trial.

We recognise the scientific value of a sham group inclusion and additionally, there is very limited published evidence using a sham control versus PRP treatment of VLS. We identified only one randomised controlled trial which compared platelet-rich plasma injections (n=19) to saline injections (n=10) (5). The authors used histology, appearance and patient symptoms as outcome measures. They showed no significant difference in all measures between groups. There are no trials with sham in AFG and no trials with sham against AFG and PRP.

We will ask patients to complete questionnaires after recruitment into the study and after surgery to assess the following:

- After randomisation: Would you have agreed to be randomised into this study if a sham procedure was included? If No, can you please explain why?

- After the surgery: Having been through the surgical procedure and general anaesthesia, would you have agreed to undergo this process if there was a chance that you would have a saline injection instead of the treatment? If No, can you please explain why?

The results of these questionnaires, in conjunction with discussions with the patient advisory board, patient representative and the co-applicants will help us decide whether a sham procedure should be included in the definitive trial from a patient perspective. We will also approach agencies like the British Society for the Study of Vulval Disease to survey clinicians and seek their opinions through formal presentations, focus groups and clinical acceptability surveys.

Pre-submission

6. Provide justification how adherence to protocol and acceptability of treatment to clinicians can be assessed with RFH as the only study centre and the only centre in the country already providing autologous fat transfer.

Acceptability

We have already engaged using a focus group with dermatologists and gynaecologists and we will further expand this work with our collaborating colleagues from Southampton, Nottingham and Norwich to assess the acceptability of the study protocol and treatments and adherence to protocol as part of our feasibility trial. Using this focus group feedback, we will develop quantitative questionnaires concerning the key themes around the acceptability or lack thereof of the future clinical trial and the acceptability of the interventions themselves to the clinical community.

We have presented our trial design proposal to the British Society for the Study of Vulval Disease as well as discussed clinical acceptability, patient recruitment, retention and sham procedure acceptability. We plan to further expand on this engagement through formal presentations, focus groups and clinical acceptability surveys.

We will collect the data on the acceptability of treatment to clinicians through formal focus groups, clinical acceptability surveys, academic conference presentations and informal discussions.

Adherence

If patients are found to be non-adherent to the study protocol, we will organise a telephone follow-up with them and assess the reasons for non-adherence to identify ways to support them and improve treatment adherence. All patients who withdrew from the study will be asked to complete a feedback survey to help us identify the causes and solutions to improve patients' experience.

The trial will be overseen by the Trial Steering Committee and the Independent Data Management Committee. Both groups will monitor the adherence to treatment and data completeness.

7. The Committee recommended further development of the patient and public involvement (PPI) component. In particular, additional details about how the PPI representatives would be involved in the study and how their views would be taken into account were requested. The team were also requested to justify the inclusion of the PPI member at only 1% FTE.

Our patient co-applicant, a female with VLS, who is an active participant in online VLS patient support groups will be a crucial part of this trial. She has been involved in the trial design and planning and has reviewed the plain English Summary, Patient Information Sheets and Consent Form. She will be a member of the Trial Management Group and will attend regular meetings to discuss the trial progression. She will be supported in her role as the Patient and Public Involvement (PPI) lead by an academic clinical fellow and consultant clinical psychologist before, during and after the meetings to ensure that she has a full understanding of the trial procedures and can make informed contributions to the discussions. In addition, the agenda for every meeting will have a permanent item for discussion of the PPI matters. Her FTE commitment has been increased to 5% to reflect the Stage 1 feedback. She will receive compensation for her time calculated at a daily rate of £120 per meeting and in addition any travel costs incurred. This has been added to the study budget.

In addition, we will seek to appoint an independent PPI member of the Trial Steering Committee who will also be reimbursed for their time.

We have recruited a trial patient advisory group (PAG) consisting of 4 women with VLS from Royal Free Hospital and Nottingham University Hospital. Patients from Nottingham have a vast experience in PPI work through their previous work on the PEARLS trial (Proactive against reactive therapy for the prevention of lichen sclerosus exacerbation and progression of disease – a pragmatic, parallel-group randomised controlled trial with embedded economic evaluation and process evaluation) (NIHR funded). One of the patients is an active member of the Vulval Cancer Awareness UK charity. The PAG has already been engaged in the trial design and will be consulted during the study progression. They will attend regular online focus groups to discuss the trial progression and improve recruitment if necessary to improve patients' adherence and experience.

Our PPI members are experts by experience. Their input will ensure that the research outputs of this feasibility study are aligned with patients' needs and priorities.

Additionally, all potentially eligible patients will be interviewed to explore the reasons for agreeing or refusing to take part in the study to further improve the trial design, identify any barriers to recruitment and seek opinions regarding improving recruitment rates. All patients will be asked about their opinion on a sham procedure after randomisation

Pre-submission

and after the surgical procedure to explore whether a sham procedure should be included in the definitive trial.

One of the feasibility aims of this trial will be to identify the primary and secondary outcomes for the definitive trial and the most suitable outcome assessment tools. Our patient representative, the patient advisory board and the study participants will be asked about the acceptability of the proposed outcomes assessment tools and the most suitable outcome measures which should be included in the definitive trial.

8. The Committee requested more information on the prevalence of vulvar fibrosis in Lichen Sclerosus (VLS) in all populations and how many patients seek treatment through primary and secondary care.

We have added detailed information on the prevalence of vulval fibrosis in VLS in the Background and Rationale Section.

The prevalence of VLS in the UK is up to 3% but universally accepted as under-reported (2,3). The incidence is highest in postmenopausal women but girls as young as 4-6 years old are affected, comprising 7-15% of cases and commonly diagnosis is delayed by 1-2 years (6,7).

The prevalence of vulval fibrosis and scarring in VLS is poorly reported but studies have reported rates as high as 64-79% in adult women (2,4) and 27% in prepubertal girls (4).

There is no national register of the number of VLS patients seeking care for VLS or vulval fibrosis through primary and secondary care.

In this study, one of the eligibility criteria includes the vulval structural changes. We will be assessing the effect of the proposed treatments not only on fibrosis but also on sexual function, impact on intimate relationships, pain, itching and quality of life which were prioritised by our original PPI work.

9. The Committee sought clarity on the progression criteria to the full trial, and requested the team consider the use of the "traffic light" system.

We have adopted the traffic light system as suggested in the Stage 1 feedback. We summarised the proposed traffic light system including the proportion of eligible participants willing to be randomised, recruitment rate per month, number of active patient recruitment sites, withdrawal rate and data completeness in Table 4 (criteria for progression to the subsequent, definitive trial). We have identified mitigation strategies for each element (Table 5). We will review these at Trial Management Group meetings and how these issues may impact on a future clinical trial.

10. The Committee requested that the plain English summary (PES) be rewritten to remove the excess jargon and acronyms and to also include an explanation of vulvar fibrosis in Lichen Sclerosus. Please note that the word limit for a PES is 450 words.

The Plain English Summary (PES) has been reviewed by our patient representative and four members of the PAG. As a result, all jargon and abbreviations have been removed and a clearer explanation of the vulval anatomy, VLS signs and symptoms and their impact on the patients' life have been added.

We also included a simple explanation of vulval fibrosis in VLS. All patients agreed that the updated PES is easy to understand and includes the most important information about the trial design.

Pre-submission

6. Detailed Research plan

Using all of the headings in the order presented below, please use this section to clearly explain your proposed research. Schematics, tables, illustrations, graphs, and other types of graphics can be embedded to clarify the research plan but they should not clutter the central narrative. Images do not count towards the overall word count but inclusion of them to overcome word limits is not permitted. Images may only be included within the 'Research Plan.' Images included in other sections will be removed from the application and not seen by reviewers.

For further information for what is required under each heading please refer to the Guidance for Applicants. **As this is the main part of your application which will be considered by the reviewing committee, you should ensure that the information is accurate, succinct, clearly laid out and provides sufficient methodological detail.**

NOTE: Applicants should aim to reserve a significant proportion of the word count for the project plan to enable methodological approaches to be fully specified.

1. Background and Rationale

2. Aims and Objectives
3. Research Plan / Methods
4. Dissemination, Outputs and anticipated Impact
5. Project / research timetable
6. Project Management
7. Ethics / Regulatory Approvals
8. Project / Research expertise
9. Success criteria and barriers to proposed work

1. Background and Rationale

What is the problem being addressed?

Vulval Lichen Sclerosus (VLS) is a chronic inflammatory skin disease affecting the vulva and anogenital region (1). The prevalence of VLS in the UK is up to 3% of adult women (2,3), and girls as young as 4-6 years are also affected (7-15% of cases) (6,7).

VLS aetiology is unknown but is believed to be a combination of genetic susceptibility and environmental factors triggering an autoimmune reaction which leads to inflammation, tissue injury, dysregulation of fibroblast and collagen metabolism (1).

VLS symptoms include itching, burning and pain. Typical lesions are flat, porcelain-white spots with erythema, ecchymosis, hyperkeratosis, and excoriations due to itching (1). Diagnosis is clinically based and supplemented by biopsy, in particular when neoplastic lesions are suspected (1). VLS has a 4-6.7% risk of vulval squamous cell carcinoma (8,9).

Like the untreated course in adults, failure to treat prepubertal VLS patients leads to progressive scarring with loss of architecture in up to 75% of adolescent cases (6). VLS symptoms in girls may also differ from the adult presentation with girls being more likely to have urinary or bowel symptoms and vulval purpura at first presentation (4).

Forty percent of women of reproductive age may be asymptomatic and are commonly misdiagnosed (9,10). Pruritus, which is the most reported VLS symptom, is more pronounced in hypoestrogenic states and could be the reason why women in the reproductive years experience milder symptoms, possibly contributing to the delay in diagnosis (9). The time from the reported onset of symptoms until diagnosis is up to five years (4,10). Many cases present late and are misdiagnosed as candida or other mild vulval disorders (10).

There is no cure for VLS and treatments are limited (1). First-line treatment involves lifelong topical corticosteroids (TCS) which are effective in reducing inflammation, development of fibrosis and change in architecture (11). Previous studies have shown that less than 67% of patients comply with treatment (12).

A meta-analysis found that TCS are effective in the acute and maintenance phases but are inadequate for reversing the scarring and loss of vulval architecture once it has already occurred (13). The prevalence of vulval fibrosis and scarring in VLS is poorly reported but studies have reported rates as high as 64-79% in adult women (2,4) and 27% in prepubertal girls (4). A delay in diagnosis of 2 years or less is associated with less scarring on presentation suggesting that early diagnosis may prevent scarring in some cases (4). Other treatments have limited efficacy evidence (13).

VLS commonly affects the fourchette, labia minora and majora, clitoral hood, perineum and perianal area. Dyspareunia caused by fourchette fissuring, erosions and fissures impacts sexual functioning and intimate relationships (14).

Two surgical treatments that might be effective in reversing VLS-related inflammation, fissuring and fibrosis, autologous fat transfer (AFG) and platelet-rich plasma (PRP), have limited evidence of clinical efficacy. There are no high-quality randomised controlled trials (RCTs) comparing these options against each other and standard care.

Why is this research important in terms of improving the health and/or wellbeing of the public and/or to patients and health care services?

Impact of VLS

The European Commission has acknowledged VLS as a high-burden, under-researched condition, highlighting insufficient funding relative to the disease burden (15). VLS causes sexual dysfunction manifesting as dyspareunia, vulvodynia, difficulty achieving orgasm, and loss of sexual interest (14,16). A 2015 British Association of Dermatologists vulval health survey involving 325 women revealed substantial daily life disruptions, including

Pre-submission

difficulties in walking (38%) and sleeping (36%). Sexual function was impaired in 84%, 79% experienced strain on personal relationships, 89% reported negative effects on emotional wellbeing with 22% reporting thoughts of suicide or self-harm. Work capacity is diminished in 39% (17).

Improving future health services and VLS care

This study addresses several of the Top 10 priorities of the VLS Priority Setting Partnership (18). If feasibility is shown, the subsequent RCT will aim to fill the evidence gaps in the surgical management of functional and structural architectural changes, symptom reduction and improvement of quality of life (QoL). High-quality prospective evidence is required to develop best practice guidelines to standardise treatment delivery.

The Royal Free Hospital is the only NHS centre providing AFG for VLS, with one centre in Nottingham having done so in the past. There is a sporadic provision of AFG and PRP privately without an adequate evidence base. There is no provision of PRP for VLS in the NHS. This study will include Nottingham as a Patient Identification Centre (PIC) and will explore the feasibility of a second treatment centre.

Review of existing evidence – How does the existing literature support this proposal?

Systematic reviews of clinical efficacy

We conducted a systematic review of PRP and AFG as VLS treatments, identifying 15 studies of mostly poor quality (level 3-4 evidence). These studies demonstrated improvement in VLS signs, symptoms, QoL and sexual function. One RCT (5), reported as an abstract, assessed PRP in VLS, finding no significant difference between the experimental and control groups. One RCT (19) reported the efficacy of PRP mixed with AFG and found significant improvements in fissures, itching, dyspareunia, QoL and inflammation. We found, as others did, that heterogeneity of outcome measures hindered meaningful analysis (13). No trials compared PRP vs AFG vs TCS alone.

Trial register

Having searched clinical trial databases, we identified two RTCs evaluating the effectiveness of PRP in VLS (20,21)(Table 1). There are no currently ongoing studies evaluating AFG in VLS.

Table 1. RCT evaluating PRP in VLS.

Study type	Study ID	Location	Intervention group	Control group	Number of participants	Primary Outcome	Secondary outcomes
RCT double blind	NCT04967170	USA	Two PRP injections six weeks apart	Sham procedure	60 (30 per arm)	Incidence of adverse events	Clinical Scoring System for Lichen sclerosus
RCT open label	NCT05364515	Spain	Four PRP injection two weeks apart and topical PRP serum in addition to topical clobetasol 0.05%	Topical clobetasol 0.05%	30 (15 per arm)	QoL measured by Skindex-29 index	Clinical Scale of Vulval Lichen Sclerosus Visual analogue pain scale Patient's global impression Investigator's global assessment Frequency of recurrences and complications

Pilot study

We carried out two studies; a retrospective open cohort study and a prospective open cohort study.

The retrospective cohort study evaluating the effectiveness of AFG in VLS (n=59) found significant improvement in the visual analogue scale for itching and a significant decrease in sexual distress.

The prospective cohort study (n=33) of patients treated with AFG (16). We found that AFG has a potential anti-inflammatory and anti-fibrotic effect (Figure 1).

Figure 1. Results of the prospective cohort study.

Prospective cohort study

33 women with VLS were treated with AFG

The outcome measures used

- Change in vulval architecture - Vulval Architecture Severity Scale (VASS)
- Symptoms - Visual analogue scale (VAS) for burning, itching and soreness
- Sexual function - Female Sexual Function Index (FSFI)
- Sexual distress - Female Sexual Distress Scale (FSDS)
- Anxiety and depression - Hospital Anxiety and Depression Scale (HADS)
- Romantic relationships - Relationship Assessment Scale (RAS)

Results of the study

- VASS - an overall improvement in the treated areas ($p<0.05$)
- VAS - a significant improvement was reported in the VAS for itching, soreness, burning ($p<0.05$)
- FSFI - a significant improvement in sexual function ($p<0.05$)
- FSDS - a significant improvement in sexual distress($p<0.05$)
- HADS - a significant improvement in the level of anxiety and depression ($p<0.05$)

Validation of additional outcome measures

In this trial, two assessment methods we have previously validated will be used: the Vulvar Architecture Severity Scale (22) and 3D Volumetric Imaging (23).

Patient and Public Involvement

In line with NIHR Patient and Public Involvement resources, the NIHR Collaboration for Leadership in Applied Health Research Toolkit and utilising the GRIPP2-SF framework (24) Patient and Public Involvement (PPI) has been present throughout the trial design and used as a keystone in this feasibility trial.

We worked with our patient co-applicant and a PPI focus group regularly to identify key themes:

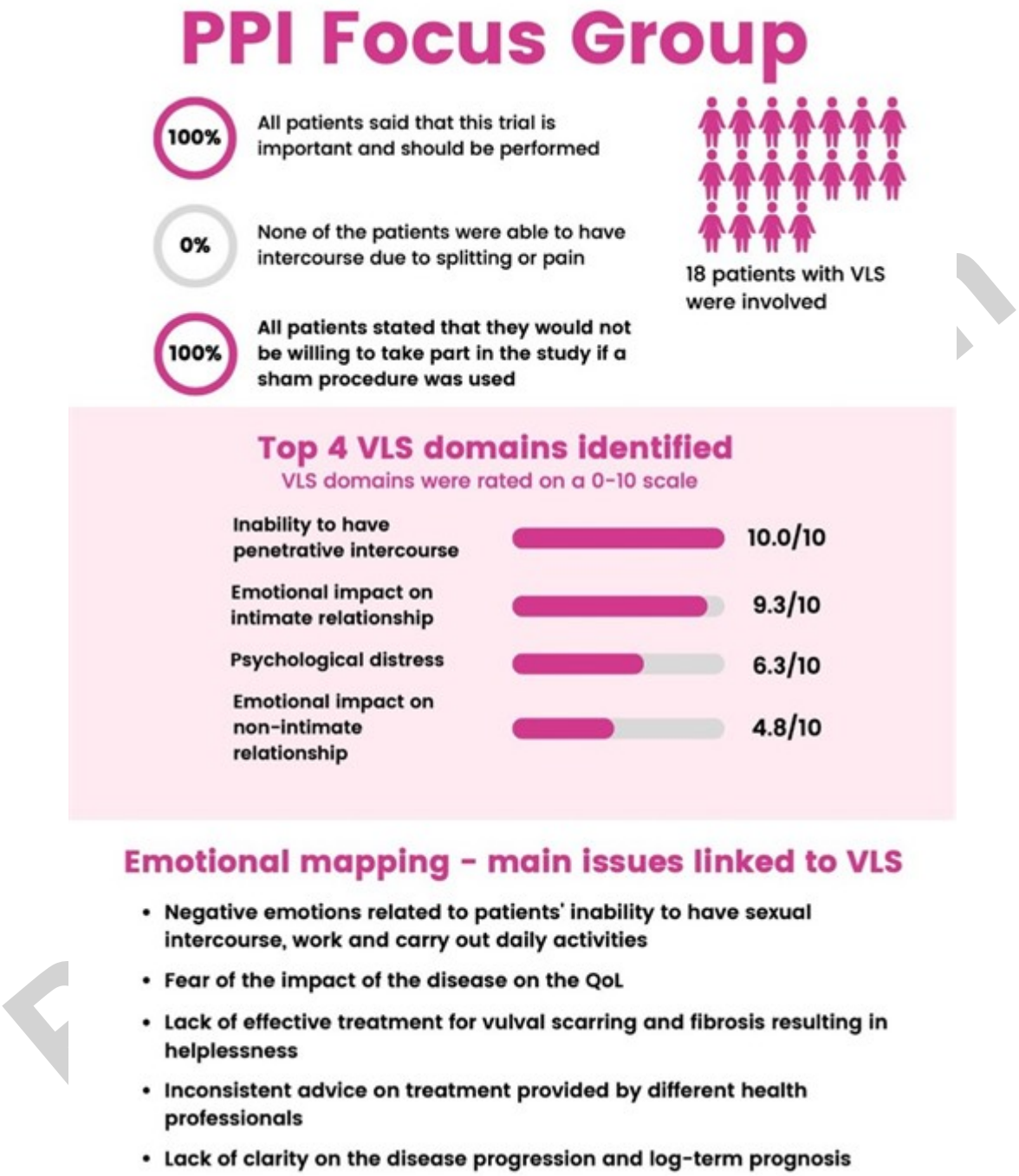
- Is present treatment adequate?
- Is this trial necessary and relevant?
- Could we include a sham?
- Barriers to entry and long-term engagement?
- What are the key outcomes we should measure?

The methods used included:

- One-to-one discussions with our patient representative and members of the patient advisory group (PAG)
- Focus group and one-to-one interviews with 18 VLS patients
- Online survey of the trial proposal and preferred outcome measures completed by 809 VLS patients
- Small focus groups and one-to-one interviews with our PPI co-applicant and members of the PAG in response to NIHR Stage 1 feedback

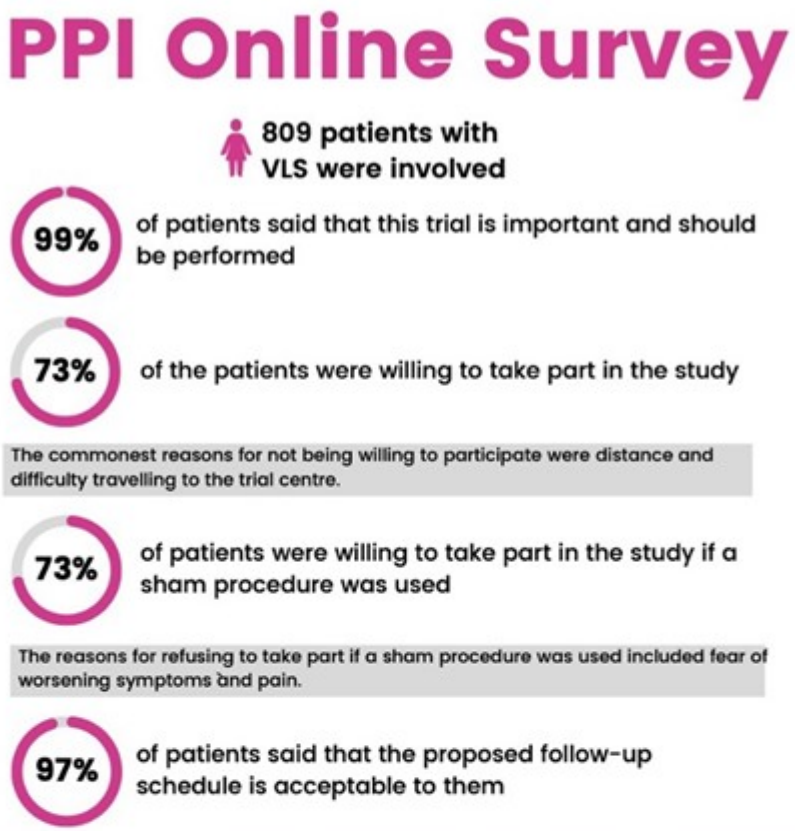
The initial PPI stage included 18 patients. Patients were invited to a focus group to discuss trial design, the proposed procedures, the sham procedure, and potential outcome measures. The outcomes tested with the group were derived from the VLS outcome domains (25). Results are summarised in Figure 2.

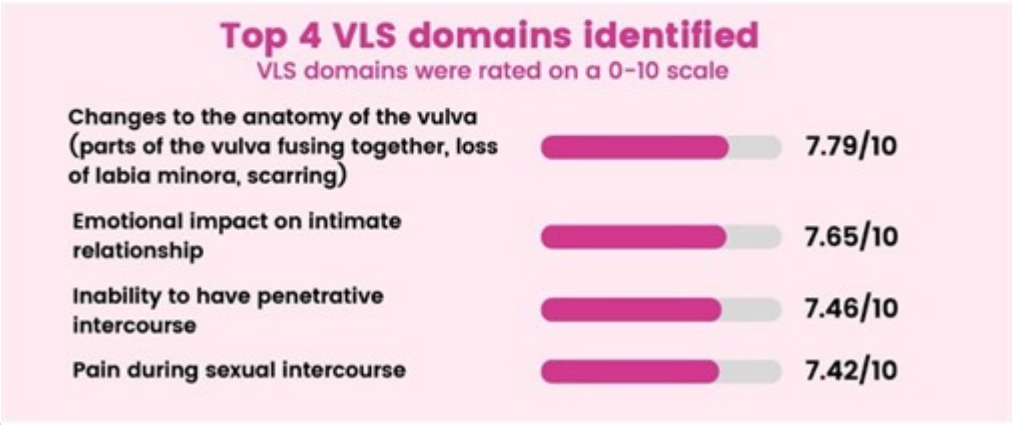
Figure 2. PPI focus group results.



The online survey was designed in collaboration with our trial consultant clinical psychologist and the patient co-applicant based on the focus group outcomes. It was disseminated on national and international VLS patient forums to learn from a wider international group of VLS patients. It was completed by 809 women. The results are summarised in Figure 3.

Figure 3. PPI online survey results.





Patients reported that addressing the changes associated with VLS would be life-changing

The survey results highlighted the need for the study and the lack of effective treatments for VLS-related fibrosis.

We are committed to involving the patients at all stages of the PETALS trial. We have involved our PPI Lead from the early stages of the trial and used her advice to shape the trial design. We also created a PAG which consists of four women with VLS to ensure that patients' views shape the progression of the PETALS trial. One of the members of our PAG is a founder of the Lichen Sclerosus and Vulval Cancer Awareness website (https://www.lsvcukawareness.co.uk) with experience in advocating for women with VLS and vulval cancer and raising awareness of these conditions. The PAG will advise us on the trial throughout its lifecycle through regular online meetings, questionnaires, results dissemination and creation of the online platform to increase awareness of the trial and the proposed treatments.

The Plain English Summary has been reviewed by our patient representative and four members of the PAG. All jargon has been removed and a clearer explanation of the vulval anatomy, VLS signs and symptoms and their impact on patients' lives have been added. All patients agreed that the updated summary is easy to understand.

2. Aims and objectives

What is the research question / aims and objectives?

Our feasibility trial will aim to establish if the progression to the definitive trial is possible. This will be achieved by answering the following questions:

1. What proportion of eligible patients are willing to be randomised?
2. What is the average number of patients we can recruit per month and the number of patients we can recruit in total?
3. What is the rate of withdrawal from the study, data collection completeness and the impact of the distance travelled to the treatment centre on the patient retention rate?
4. Are the treatments acceptable to the patients and clinicians?
5. What is the adherence to the trial protocol and adequate timing of outcome assessments?
6. What is the ideal number of centres required nationally? Can a second treatment centre be set up in Nottingham?
7. What is the optimal measure of efficacy for a multicentre RCT?
8. What is the rate of adverse events?
9. Should a sham procedure be included in the control group in the definitive trial? Is it acceptable to the patients and clinicians?
10. What is the feasibility of collecting quantitative and health economics data and analysis of cost-effectiveness?
11. What is the best way to disseminate the results among clinicians and raise awareness of the study and the proposed interventions?
12. How to disseminate the results among the patients and increase awareness of the study and the proposed procedures? What online platform should be used for communicating with patients?

3. Detailed Research Plan and Methods

Study design

This will be a single-blind (blinded assessors) randomised controlled feasibility trial (Please see Appendix_CONSORT flow diagram). Following confirmation of eligibility and written informed consent, participants will be randomised using a centralised online randomisation system (https://sealedenvelope.com/) to ensure randomisation concealment and minimise bias. Randomisation and allocation will be overseen by the trial coordinator who will not be involved in the recruitment, treatment or assessment. All patients will be started on a standardised TCS (Standard of care - 'SoC') and vulval care regimen at the baseline visit after randomisation.

Randomisation will be 1:1:1 to three treatment arms:

1. SoC alone
2. AFG + SoC
3. PRP + SoC

The number of patients recruited per month will be one of the feasibility outcomes. We plan to recruit a maximum of 75 patients. We outlined the target recruitment per month in Table 4 and the Green criteria has been set as at least 3 patients per month.

Patients in two surgical arms will undergo a procedure under general anaesthesia where AFG and PRP are collected and either AFG or PRP is injected into the vulval area. Patients will be blinded to the injection they receive.

We plan to use unmodified PRP and AFG prepared to a standard protocol injected intraoperatively with the MHRA who informed us that if used this way, PRP and AFG are acceptable for clinical use and are not Investigational Medicinal Products (IMP).

A pragmatic approach of allowing patients to continue a potent TCS which they are known to tolerate will be used, as agreed with dermatology (RS). A maintenance regimen of twice weekly (non-consecutive days) will occur. If patients experience a flare-up of VLS signs and symptoms an increase will be advised (TCS daily for seven days) as standard practice.

We will offer a Consultant Dermatologist review as adjuvant management if clinically indicated by the research team to manage patients whose disease is poorly controlled.

All patients will be followed-up for 6 months.

Health technologies being assessed

AFG is a minimally invasive surgical procedure. Adipose tissue is harvested from the abdomen with a liposuction cannula, centrifuged and injected into VLS-affected areas.

Pre-submission

PRP is obtained from a blood sample which is centrifuged to isolate platelet-rich fraction and injected into the VLS-affected areas.

Difference between current and planned pathways

Patients in the control arm will continue SoC (standardised TCS regimen), the same as the current NHS SoC. The interventions include AFG and PRP. AFG is a routine practice at Royal Free Hospital for VLS patients but is not common practice nationally. PRP is currently not offered for VLS in the NHS.

Setting

Eligible patients will be identified at Royal Free, Nottingham, Southampton and Norwich. The external centres will refer patients to Royal Free Hospital where they will be randomised, treated and followed-up. To reduce the risk of slow study progression due to NHS waiting times, patients will be operated through a waiting list initiative which will be factored in the excess treatment costs. To optimise the recruitment rate, Nottingham Hospital will act as a second Patient Identification Centre (PIC) and refer patients for treatment and follow-up at Royal Free Hospital. We will also receive referrals from Southampton through Dr Helen Lowery and Dr Alice Plant (Dermatology) and from Norwich through Dr Karen Gibbon (Dermatology).

We will increase awareness of the trial among patients through our activity on the online VLS patient support groups.

We will continue to collaborate with female vulval health agencies like the British Society for the Study of Vulval Disease (BSSVD) (Please see the Appendix_BSSVD Support Letter) and the Wellbeing of Women (WoW) to increase awareness of the study among healthcare professionals.

Summary of patients as research participants

Target population

We consulted the NIHR INCLUDE Guidelines (26) to ensure the eligibility criteria allow the inclusion of underserved patients. We will record age, ethnicity, educational level, employment status and other indicators of underserved patients.

Inclusion criteria

- Adults (>18 years)
- Female sex
- Patients who live in the UK
- Patients with expert-diagnosed clinical and/or histological diagnosis of VLS
- Able to give informed consent
- Able to communicate in English
- Structural changes secondary to VLS

Exclusion criteria

- Active malignancy or previous diagnosis of intraepithelial neoplasia/carcinoma of the vulva
- Previous vulval AFG or PRP or vulval surgery
- Body mass index <18.5 or >35.0
- Contraindications to general anaesthetic
- Active infection that precludes fat harvest, AFG or PRP injection
- Pregnancy, breastfeeding or planning for pregnancy during the study period
- Connective tissue and autoimmune diseases and diseases which affect the vulva

Screening, eligibility assessment and consent

Trained members of the research team and clinical collaborators on the Delegation of responsibility log at Royal Free Hospital, Nottingham University Hospital, Southampton and Norwich will identify eligible participants attending Dermatology and Vulval clinics and inform them about the trial. All potentially eligible patients who express initial interest in taking part will be given Patient Information Sheets and Consent Forms. This will be followed by a phone call from a member of the research team to discuss the study details and confirm their interest and eligibility.

If eligible and willing to take part, potential participants will be invited to attend a clinic appointment at Royal Free Hospital. All patients will be given time to ask questions before they provide written informed consent witnessed by a member of the study team. They will then be randomised into one of the trial arms and undergo the baseline assessments.

Pre-submission

A copy of the signed consent form will be given to the patient, another placed in the patient's notes, and the researchers will retain a further copy.

If patients do not consent to take part in the study, they will be asked to complete a feedback form to help us understand barriers to recruitment.

Patients randomised to the surgical arms (PRP or AFG) will undergo additional baseline assessments on the day of surgery.

Follow-up

Participants will be followed-up at Royal Free Hospital. They will be able to discuss any urgent queries with a trained clinical research team member via the PETALS trial helpline. We will allocate a member of staff to reply to patients' queries. This service will be available from Monday to Friday and will help us to work closely with our patients.

Participants in the AFG and PRP arms will be contacted by a research team member via telephone 7 and 14 days postoperatively to monitor for any complications and offer advice if necessary. All patients will attend follow-up visits at 3 months and 6 months. Patients will be reimbursed for travel. Each follow-up visit will last approximately one hour. A summary of assessments made at each visit is presented below in Table 2.

All patients will be on topical corticosteroids and the frequency of use will be monitored through patient diaries and assessed during the follow-up. We will explore the use of electronic reminders and patient diaries to assess which format works best for patients and maximises data completeness. This will be explored with the PPI lead and PAG members.

Table 2. A summary of assessments made at each visit.

	Duration	Baseline	3 months	6 months
Acceptability of the proposed treatments	2 mins	x	x	x
Adherence to the standardised topical corticosteroid regime and flare ups	5 mins	x	x	x
Patient questionnaires and Health Economic questionnaires	20 mins	x	x	x
2D and 3D digital photography	5 mins	x	x	x
Measurement of the vulval microcirculation (laser speckle contrast imaging and visual capillary microscope)	10 mins	x	x	x
Measurement of the vulval skin fibrosis (fibrometer, cutometer, durometer, high frequency ultrasound)	10 mins	x	x	x
Physical Examination	10 mins	x	x	x
Adverse events	-	x	x	x
Acceptability of sham procedure	5 mins	x	x	x

Data collection

A formal Data Management Plan will be constructed to describe the procedures involved in the data management activities and processes for the study so that it is managed following the Good Clinical Practice guidelines, standard operating procedures (SOPs), appropriate regulatory requirements and the study protocol.

Data collected during the trial is summarised in Table 3 below. Source data will be in the form of, but not linked to, patient medical records and study-specific care report forms. This will be stored at Royal Free Hospital as encrypted and anonymised password-protected files stored on NHS computers. Trial-specific data will be entered into the database by a dedicated member of staff at Royal Free Hospital and this will be verified by the Trial Manager. All paper forms will be stored in a locked cabinet in a designated office at Royal Free Hospital and will only be accessed by authorised trial personnel.

Screening logs will be kept at each recruitment site and will be sent to the Royal Free Hospital periodically to tabulate the information.

The software model for the data entry and monitoring will be REDCap, which is a secure, web-based platform providing an interface for validated data capture, audit trails for tracking data manipulation and export procedures. During the trial, Reporting Data Extracts will be provided for safety reporting, independent Trial Steering Committee meetings, interim and final analysis, and other reporting periods as outlined in the Statistical Analysis Plan. Only pseudo-anonymised data will be shared under the terms of the consent forms and will be available to users under a data-sharing request. After analysis, all study data will be stored and archived according to the Sponsor's SOPs for up to 10 years.

Table 3. Summary of data collected during this feasibility study.

Pre-submission

Data collected	Screening for eligibility	Baseline visit	7- and 14-days post-op	3 months	6 months	Throughout the study
Recruitment information	X	X				
Reasons for unwillingness to participate	X					X
Acceptability of the proposed treatments						X
Adherence to the standardised topical corticosteroid regime and flare ups						X
Patient questionnaires and Health Economic questionnaires		X		X	X	
2D and 3D photography		X		X	X	
Measurement of the vulval microcirculation (laser speckle contrast imaging and visual capillary microscope)		X		X	X	
Measurement of the vulval skin fibrosis (fibrometer, cutometer, durometer, high frequency ultrasound)		X		X	X	
Physical Examination		X		X	X	
Adverse events						X
Willingness to participate if a sham was used		X		X	X	
Most appropriate way to disseminate the results among patients and increase awareness of the study						X

Statistical Analysis Plan

A CONSORT diagram will be produced to describe the flow of patients.

All feasibility outcomes will be reported using descriptive statistics. Continuous endpoints will be summarised for baseline characteristics using means and standard deviations (medians and interquartile ranges) per arm. Categorical and binary variables will be summarised with frequencies and percentages per arm.

Surgical complications will be reported using the Clavien Dindo scale with descriptive statistics. Adverse events will be tabulated by treatment arm with frequencies and percentages.

The efficacy endpoints will be analysed with linear mixed models to provide the differences in means with 95% confidence intervals between the surgical and control arms at the two follow-up time points, adjusted for baseline.

Summaries of the regular data reviews will be shared with the oversight committees.

Health Economics Analysis

The feasibility study will aim to plan the economic evaluation within the definitive RCT, aiming to estimate the cost and the potential economic benefit for the NHS, personal and social services. We will explore the feasibility of collecting data to assess the costs for participants and families from a societal perspective.

Within the feasibility study, we will:

- Identify the main cost components of each surgical intervention (equipment, materials, staff cost)
- Identify the potential costs of treating side effects following surgical interventions
- Collaborate with the PPI group to adapt the Client Service Receipt Inventory for the study to collect data on participants' health and social care resources
- Identify the main costs for participants and families (e.g., productivity losses)
- Assess participants' Health-related Quality of life and calculate Quality-Adjusted Life Years (QALYs)
- Perform a literature search to identify economic studies and evidence available on similar interventions (e.g., to capture data that might be needed in addition to those collected within the feasibility trial)
- Design a decision model (e.g., Markov model) using data from the feasibility study to describe the participants' pathway in the three treatment arms and to model costs and effects beyond trial based on up-to-date UK sources, published literature and clinicians and PPI representatives' opinions

In the definitive RCT, a model-based approach will be used to assess cost-effectiveness.

We will report descriptive statistics of EQ-5D-5L index values, QALYs, cost of surgical interventions and healthcare resource use per participant per group. We will perform a preliminary analysis of the cost-utility of surgical interventions against control arms for VLS to inform the definitive trial. We will report the mean incremental cost per QALY gained.

Measurement of outcomes

Feasibility outcomes

1. The proportion of eligible patients willing to be randomised **(the primary feasibility outcome).**
2. The average number of patients recruited per month and the total number of recruited patients.
3. Rate of withdrawal from the study, data collection completeness and the impact of the distance travelled to the treatment centre on patient retention rate.
4. Acceptability of the proposed treatments to patients and clinicians.
5. Adherence to the trial protocol and adequate timing of outcome assessments.
6. Feasibility of setting up a second treatment centre in Nottingham and evaluation of the number of centres required nationally to run a multicentre trial.
7. The appropriate outcome assessments and their timing. Investigated measures will include:
    a. Patient-reported signs and symptoms – Vulvovaginal Symptom Questionnaire (VSQ), Visual assessment score (VAS) for soreness, itching, burning, dyspareunia, The Female Genital Self-Image Scale (FGSIS), Clinical Lichen Sclerosus Score (CLISSCO) subjective part, The Vulvar Pain Assessment Questionnaire (VPAQ), Pain Anxiety Symptom Scale Short Form 20 (PASS-20), Genital Appearance Assessment Scale (GAAS)
    b. Patient-reported sexual function - Female sexual function index (FSFI), Female Sexual Distress Scale (FSDS)
    c. Patient-reported psychological status and QoL questionnaires - Vulvar quality of life index (VQLI), Dermatology Life Quality Index (DLQI), Skindex-16, Short Form Survey 12 (SF-12), Hospital Anxiety and Depression Scale (HADS), Relationship Assessment Scale (RAS), Clinician-reported outcomes – Vulvar Architecture Severity Scale (VASS), CLISSCO objective part, Clitoral phimosis, Interlabial sulci involvement, Vulvar introitus narrowing (CIV) score
    d. Imaging - 3D Vectra h1 & 2D photography, measurement of microcirculation by laser speckle contrast imaging and visual capillary microscope (HVC)
    e. Skin fibrosis - FibroMeter, cutometer, durometer, high-frequency ultrasound
8. Rate of adverse events.
9. Acceptability to include a sham intervention to patients and clinicians in a future randomised prospective trial.
10. Feasibility of collecting health economics data and preliminary estimates of cost-effectiveness using:
    - Healthcare resource utilisation – Client Service Receipt Inventory (CSRI)
    - Health-related QoL – EQ-5D-5L
    - Impairments in work and activities - Work Productivity and Activity Impairment (WPAI)

11. The most appropriate way to communicate with patients to disseminate the results of the study and increase awareness of the proposed treatments and the definitive trial.

Pre-submission

12. The most appropriate way to communicate with clinicians to disseminate the results of the study and increase awareness of the proposed treatments and the definitive trial.

All assessment methods are non-invasive. All questionnaires used in this study have been previously validated and are widely used in trials investigating VLS. We have reviewed all the questionnaires with our psychology co-applicant (EH) to assess their suitability for inclusion.

A search of the COMET database revealed a lack of finalised VLS core outcome set. However, core domains have been established (27). The choice of outcome measures used was based on PPI, clinical expertise, a systematic review of published outcome measures, our prospective cohort study (16) and VLS domains (25).

Safety outcomes

Safety monitoring will include any adverse events related to the treatments. In addition, the Clavien Dindo Scale will be used to assess the severity of surgical complications. Our literature review and clinical experience suggest that both interventions are safe with minor adverse events reported including bruising and pain.

If serious adverse events are identified, this will be investigated immediately and discussed by the chief investigator with the Trial Management Group (TMG), Trial Steering Committee and the Sponsor within 1-2 working days.

TMG will also be regularly updated on any non-severe adverse events and complications.

Sample size

Minimum target recruitment of 3 patients per month has been estimated as outlined in the traffic light system (Table 4). This has been set as a minimum number for the outcome of this feasibility trial with a number of mitigations to achieve and exceed this minimum recruitment. We plan to recruit a maximum of 25 patients per group which would result in a total sample size of 75.

We illustrate the power that this sample (n=75) might provide based on the prospective cohort study (n=33) of VLS patients treated with AFG (16). Using the Female sexual function index (FSFI), the standard deviation of the pre-post delta in FSFI is ~ 5 units with a mean of 4 points for the delta. If the difference in treatment effect between SoC and one of the surgical arms in the study is 4 points, assuming the same standard deviation, a sample size of 25 per arm provides 80% power at the conventional 2-sided alpha of 5%. This assumption and analysis will also be used to estimate the correct future sample size as well as determine the appropriateness of this outcome measure in a future multi-centred trial.

Recruitment rate

Eligible participants will be identified at Royal Free, Nottingham University Hospital, Southampton and Norwich Dermatology departments. Approximately 500 patients are seen in the Vulval clinics at Royal Free every year and around 380 of these are new referrals. There are around 200 new VLS patients seen at Nottingham University Hospital and 350 in Southampton each year. Participants will also be recruited through posters, leaflets, scientific societies and patient forums.

We plan to recruit a minimum target recruitment of 3 patients per month (Table 4) and up to a maximum recruitment of 25 patients per group.

4. Dissemination, outputs and anticipated impact

What do you intend to produce from your research?

The results will inform the design of a definitive trial aiming to assess the efficacy of AFG and PRP as treatments for VLS. This feasibility trial will allow us to assess the primary and secondary outcomes and the most suitable assessment tools, calculate the sample size for an adequately powered trial and decide on the number of required treatment centres. We will also assess the feasibility of performing quantitative and health economics data analysis.

We will identify any barriers to setting up more treatment centres.

We will also assess the acceptability of the trial design to patients and clinicians, in particular the acceptability of including a sham arm.

Pre-submission

One of the feasibility outcomes will be to assess the best way of communicating with patients to disseminate the results of the study and increase awareness of the trial and proposed procedures. We will set up an online platform of patients' choice.

We will present the outcomes of this feasibility study at national and international academic dermatology, gynaecology and plastic surgery conferences.

How will you inform and engage patients/service users, carers, NHS, social care and the wider population about your work?

We presented the protocol of this feasibility study at the British Society for the Study of Vulval Disease (BSSVD) national meeting in March 2023 to assess the acceptability of the study design and increase clinicians' awareness about the study. We will use the same process to inform the clinicians about the definitive trial design and ask how to best disseminate the results among healthcare professionals.

We have also developed a relationship with the Wellbeing of Women (WoW) charity and will liaise with vulval disease and female health charities to further raise awareness of the trial. We will work with the NIHR Communications team to maximise our impact.

We will engage clinicians through presentations at national academic conferences and publications in peer-reviewed journals.

We have already engaged with VLS patients through online questionnaires published on the VLS national and international patient support groups composed of 11.2K members. The questionnaires were completed by 809 and most patients (99%) thought that the study design was acceptable and that this study was important. As a part of this study, we will develop an evidence-based way of informing the patients about the outcomes of the study which will be used to engage patients and increase their awareness of the study.

All study participants will be contacted by their preferred means of communication with the results of the trial and will be asked to provide feedback on the study to help us identify ways to further improve the trial design and patients' experience as participants.

How will your outputs enter our health and care system or society as a whole?

The results of this feasibility study will provide the information required to set up a definitive trial which in turn may influence the national guidelines. After the completion of this feasibility study, we will work with female vulval health agencies, patient support groups and our clinical collaborators to disseminate the results of the study. We will use this multisource feedback to design a robust trial. We will work on setting up other treatment centres for the definitive trial and streamlining the referral process.

What further funding or support will be required if this research is successful (e.g. from NIHR, other Government departments, charity of industry)?

If this feasibility trial is successful, we will apply for the NIHR Health Technology Assessment funding for the definitive trial as well as for the grants offered by the WoW and LifeArc charities.

What are the possible barriers for further research, development, adoption and implementation?

The barriers to research include the funding and a limited number of agencies we can approach as VLS is an under-researched high-burden disease.

The barriers to the adoption of the proposed procedures include the need for training and a limited number of practitioners able to currently provide the treatments. We further assess this in this feasibility trial and identify ways to share our expertise and set up other treatment centres.

The widespread implementation of the proposed treatments may be limited by the level of evidence produced by our study which will depend on our ability to recruit enough patients to evaluate the effectiveness of the treatments. It will also rely on the commissioning groups such as local Care Quality Commission groups and the National Institute for Health and Care Excellence.

What do you think the impact of your research will be and for whom?

Patients: The treatment options for VLS are very limited, especially for VLS-related structural changes. If AFG or

Pre-submission

PRP are shown to be efficacious in improving VLS signs and symptoms, they may be implemented as a standard surgical treatment. Additionally, sexual dysfunction and VLS-related changes are significant mental health burdens on the patients and identifying new effective treatments may result in improved quality of life. Data gathered in our prospective and retrospective studies evaluating AFG in VLS has highlighted that addressing these issues would be life-changing to patients and would positively impact their self-esteem, intimate relationships and quality of life.

Public wellbeing and Health economics analysis: Severe VLS is associated with a significant functional impairment affecting patients' sleep, daily activities, and work. If the VLS symptoms were reduced for these patients, society would benefit due to a reduction of societal opportunity costs from the loss of productivity and unemployment. This feasibility study will allow us to plan the economic evaluation within the definitive RCT, aiming to estimate the cost and potential benefits for the NHS and personal and societal services. This study may also improve VLS awareness among the public and might help get funding streams for female vulval disorders.

How will you share the progress and findings of your research with study participants?

Participants will be contacted by email or post, as per preferences, to inform them of trial results. Results will be disseminated on social media patient support groups. Trial result information will be created with the PPI co-applicant and the patient advisory board. Written information, infographics, and videos will be produced to illustrate the trial results. During the trial, we will update participants through the online support groups.

5. Project/ Research timetable

Referring to the attached Gantt chart and participant flow diagram (Appendix_Gannt Chart and Appendix_CONSORT flow diagram) :

Project timetable:

Total duration: 2 years and 10 months.

- 6 months set-up
  - Research Ethics Committee and Health Research Authority approvals
  - Registration on public databases
  - Study launch meeting
- 16 months recruitment
  - the recruitment will be started at the Royal Free Hospital first and then rolled out to other sites.
- 6 months follow-up and data collection
- 6 months analysis and write-up, dissemination, development of the protocol for the definitive trial, submission of the final report

The PPI, active networking and coordination of the research community will take place throughout the study.

6. Project Management

The Royal Free will sponsor this study, employing a trial manager who will report to the Sponsor. The statistician, health economist and database manager are University College London (UCL) Comprehensive Clinical Trials Unit (CCTU) employees. Professor Peter Butler (Chief Investigator) will take overall responsibility for the organisation of this trial, including its running and completion within the allotted time frame and budget. As Chief Investigator he will ensure that the appropriate local and national approvals have been obtained before enrolling the first participant. He will be assisted by an experienced team of co-applicants from Royal Free Hospital, Nottingham and UCL. This cost-effective approach will lay a strong foundation for conducting the subsequent definitive RCT through a Clinical Trials Unit.

Several members of the team have considerable experience managing feasibility randomised controlled surgical trials, and have been involved in the design, development and writing of this application, providing statistical, trial management, regulatory and costing advice throughout. Members of the team will also be responsible for data management, trial set-up and closure, statistical monitoring, and data analysis.

Early in the project, a launch meeting will be held to allow for detailed discussions with all staff to facilitate efficient site set-up. The day-to-day management of the trial will be carried out by the team at the Royal Free Hospital London and Nottingham. Data will be collected on a bespoke database (REDCap) kept in a secure environment and backed up regularly. When data collection is complete, this data will be analysed by the trial statistician according to a pre-approved Statistical Analysis Plan.

The Trial Management Group (TMG) will consist of the co-applications and will be chaired by Professor Butler. They will meet regularly (initially weekly) via teleconference (Teams). The PPI co-applicant will be given a briefing before each meeting to ensure they can understand the terminology, enabling them to contribute effectively.

Pre-submission

Independent oversight will be provided by a Trial Steering Committee (TSC) and will consist of people with appropriate expertise and experience. They will meet by teleconference (Teams) before the first participant is enrolled, yearly, finishing with a final meeting to discuss the outcomes and analysis. More frequent meetings may be required if issues arise that need to be discussed, such as changes to the protocol to improve recruitment.

One member of the TSC will be nominated to review safety data and report to the Committee. Any safety concerns raised during the trial will be investigated by Professor Butler and communicated to both the TSC and Sponsor (Royal Free Hospital London).

7. Ethics/ Regulatory approvals

If the trial funding is granted, the trial protocol will be finalised, and the application submitted to the ethics committee before the funding has commenced to avoid potential delays. As described in point 1 of the Response to Stage 1 section, the MHRA's approval is not required for this feasibility trial.

8. Project/ Research Expertise

Professor Norman Williams is a methodologist with more than 25 years of experience running clinical trials at UCL. He specialises in "surgeon-led" studies, particularly feasibility randomised trials with a pragmatic design. He has authored or co-authored more than 300 publications and teaches trial methodology to MSc degree students. In this study, he will be involved in the database design, study set-up, coordination of the trial and its conduct and management, and analysis and write-up.

Dr Hakim-Moulay Dehbi is head of statistics at the Comprehensive Clinical Trials Unit at UCL. He has a wealth of experience in clinical trials in a variety of disease areas, including cardiovascular diseases, ophthalmology, oncology, infectious diseases and neonatology among others. His main methodological interest is the design of early-phase clinical trials. He has published extensively on this topic and is well-cited.

Miss Monica Panca is a senior health economist at the Comprehensive Clinical Trials Unit at UCL. She is involved in numerous clinical trials in different disease areas including mental health, cardiovascular diseases, neonatology, and ophthalmology. Her methodological interest is in decision modelling using different modelling techniques. She has published extensively in peer-reviewed journals.

Dr Esther Hansen is a Consultant Clinical Psychologist and Trust Lead for Psychological Therapies, with 20 years of clinical experience of working with patients with long-term health conditions or those undergoing surgery. She provides psychological interventions to patients with vulval lichen sclerosus, to help with symptom management, and improve their quality of life, emotional wellbeing and sexual functioning. She has expertise in running patient focus groups, qualitative analysis and engaging patients as part of service development and in research. She has published in peer-reviewed medical and psychology journals.

Miss Aurora Almadori is a Plastic Surgeon with a specialist interest in Vulval Reconstruction and Regenerative Surgery. She has recently completed a PhD on the use of fat grafting to treat dermal fibrosis and scarring at UCL. She has received 14 prizes and awards for her research. Miss Almadori has published more than twenty scientific publications in peer-reviewed journals and has contributed to five textbooks of plastic surgery.

Rosalind is a clinical academic with a specific interest in women's health and vulval disease. She is an NIHR Advanced Fellow and current projects revolve around the accurate diagnosis, long-term treatment, and core outcomes in lichen sclerosus. In particular, she is the lead for the NIHR HTA-funded 'PEARLS' multicentre randomised controlled trial. Rosalind is the research lead for the British Society for the Study of Vulval Disease, a Fellow of the International Society for the Study of Vulvovaginal Disease and co-lead for the Patient Advisors for Vulval Research (PAVE) group in Nottingham.

Dr Victoria Swale is a Consultant Dermatologist and Dermatopathologist at Royal Free Hospital with 20 years of experience. She specialises in vulval diseases. Dr Swale is a member of the British Society for the Study of Vulval Disease, the British Society of Dermatologists and the Honorary Secretary of the British Association of Dermatologists.

Dr Benjamin Langridge is a Specialist Registrar in Plastic Surgery at the Royal Free Hospital. He has extensive clinical experience in the use of AFG and PRP in the treatment of fibrotic diseases, including VLS. He has completed an NIHR Academic Clinical Fellowship, MSc degree, and his PhD research on the treatment of fibrotic diseases with AFG. He has published widely and co-ordinates the NIHR-funded Sys-Stem feasibility study investigating AFG as a treatment for orofacial fibrosis in Systemic Sclerosis.

Pre-submission

Ms Sara Jasionowska is a plastic surgery trainee at Royal Free Hospital. She has led the PPI component of this study and has carried out literature reviews of the outcome assessments in VLS and the effectiveness of AFG and PRP. She is a co-applicant on the NIHR-funded HTA GRACE trial where she was responsible for liaising with peripheral sites and the PPI work.

9. Success criteria and barriers to proposed work

What contingencies you will put in place to reduce or eliminate each risk or its impact

We identified potential barriers to progression to the definitive trial and summarised them in Table 4 below.

Table 4. Criteria for progression to the subsequent, definitive trial. This table is for guidance only as measures may be taken during the feasibility trial to improve the situation (e.g. to increase the recruitment rate).

Progression criteria	Red	Amber	Green
The proportion of eligible participants willing to be randomised	>30%	30-50%	>50%
Recruitment rate per month	1-2 patients	2-3 patients	3 or more
Number of active patient recruitment sites	0	1	2 or more
Withdrawal rate	>40%	20-40%	<20%
Data completeness	<60%	60-80%	80-100%

We will carry out interim assessments of the proposed criteria to progression to the definitive trial and will discuss any potential issues during the regular TMG meetings. We also identified potential solutions if any of the identified progression criteria fall under the red or amber category which are summarised in Table 5.

Table 5. Potential actions which will be taken if the identified progression criteria fall under the red or amber category.

Pre-S

Progression criteria	Potential actions
The proportion of eligible participants willing to be randomised	- Patients will be asked about the reasons why they are not willing to be recruited and their feedback will be discussed by TMG and PAG. - If any changes in the trial design need to be made, this will be discussed with TSC.
Recruitment rate per month	- Reasons for poor recruitment will be discussed with eligible patients and if any changes need to be made this will be discussed with TMG and TSC. - We will liaise with female vulval health agencies and online patient support groups to increase the awareness of the trial.
Number of active patient recruitment sites	- We have already included Nottingham University Hospital, Southampton, and Norwich as patient identification sites. - If more referring centres are required, we will identify further recruitment sites.
Withdrawal rate	- Feedback from patients who dropped out from the study. - The feedback from the patients who withdrew from the study will be discussed with the TMG and PAG. - This will be discussed with TSC if any changes in the trial design are required.
Data completeness	- Regular assessments of the data completeness will be made. - Patients will be phoned to gather missing data. - Patients will be asked for their feedback on the reasons for data incompleteness and the study design may be altered based on this.

Safety outcomes will be monitored throughout the study. If any issues are identified, the Principal Investigator will notify the TMG and TSC immediately. For further details please see the Safety Outcomes section.

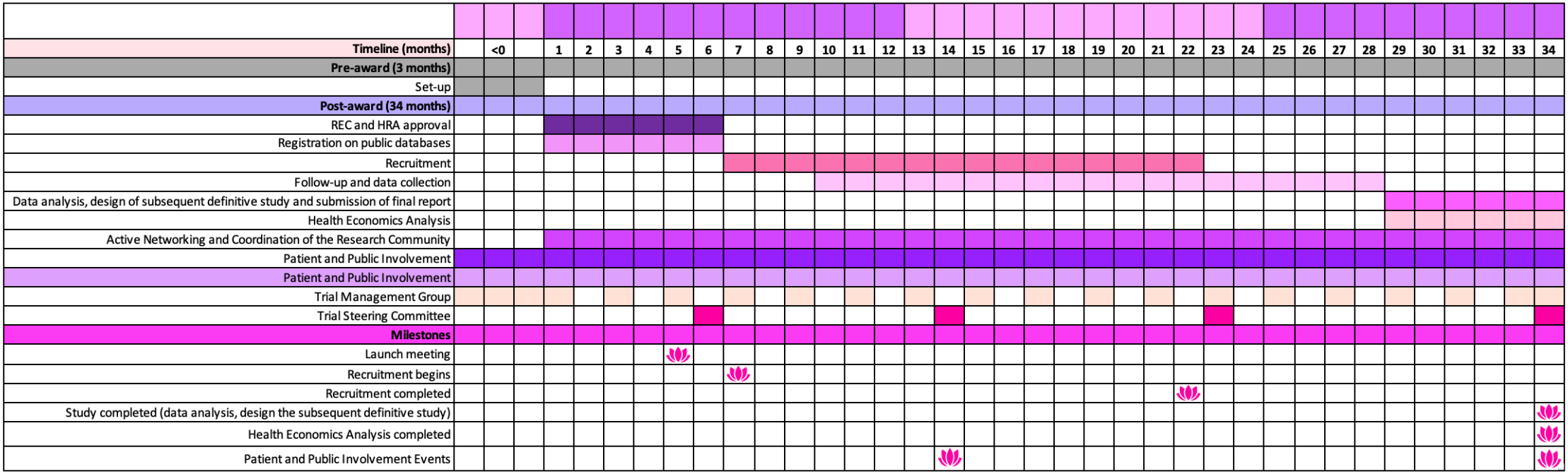
Design of the subsequent definitive trial

The protocol for the definitive trial will be developed during the last six months of this feasibility trial. The definitive RCT will be designed by an advisory group encompassing all co-applicants, patient representatives and clinical collaborators. Regular meetings will serve to address the following: whether a definitive trial should be conducted, the primary and secondary outcomes, sample size and estimated treatment effect (based on the results from this feasibility trial), the inclusion of a sham procedure, follow-up schedule, and the minimum number of centres required to recruit the calculated sample size.

Pre-submission

Pre-submission

PETALS trial Gantt Chart



7. Patient & Public Involvement

Please describe how patients and the public have been involved in developing this proposal.

The PPI activity during the trial design phase included one-to-one interviews, focus groups and online surveys. We created a PAG after NIHR Stage 1 feedback. Members of the PAG have a lived experience of VLS and lipotransfer surgery. Please see the Patient and Public section of the Detailed Research Plan for further details of the PPI approach used during the trial design phase.

Our patient co-applicant will be the PPI lead for this study and a member of the TMG. She will coordinate and chair regular PAG meetings and attend the TMG meeting throughout the study duration. Her involvement will be reimbursed as per a standard PPI rate at Royal Free Hospital of £120 per day and the cost of a day return tube ticket. All members of the PAG will be reimbursed for their involvement in the online meetings.

The data collected through online questionnaires, patient feedback from interviews held during their follow-up visits and feedback from the patient representative and PAG meetings will be analysed and collated by the study academic fellow and the reports will be presented during the regular TMG meetings. Patients' views obtained through these regular PPI activities will be used to directly shape the trial throughout its lifecycle. It will be used to improve the trial design, the recruitment and retention of study participants, identify outcome measures for the definitive trial, decide whether a sham procedure should be included, how to disseminate the results and communicate with the patients and public. It will be a keystone in designing the definitive trial.

Participants of the study will be interviewed during their following visits and asked to provide feedback on their experience as a trial participants. If any areas of improvement are identified, we will discuss these with the TSC and TMG to further improve the trial design.

All PPI members will be supported by the clinical academic fellow and our psychology co-applicant throughout the study duration to ensure that they understand the trial progression required to make informed decisions and actively contribute.

Please describe the ways in which patients and the public will be actively involved in the proposed research, including any training and support provided.

Please see the PPI section of the Detailed Research Plan for further details of PPI activities embedded in the trial design phase. The PETALS trial will be co-produced by VLS patients. Our patient representative will be the PPI Lead and a member of the TMG. She will attend regular meetings to discuss the progression of the trial and troubleshoot any issues. Our PAG composed of four women affected by VLS will meet online regularly to discuss possible changes to the design of the study, the most suitable outcome measures for the definitive trial, the inclusion of a sham procedure, the most appropriate platform to disseminate the results and communicate with a wider group of VLS patients.

Members of the PAG and the patient representative will review the Patient Information Sheets and Consent Forms and will help us produce patient educational materials explaining the trial design and the proposed treatments. They will be consulted if any barriers to recruitment or patient retention are identified to identify potential solutions.

We will ask all eligible patients who declined participation to complete a questionnaire on the reasons for their unwillingness to participate to identify possible areas of improvement. We will also contact all patients who dropped out of the study and ask for their feedback. Participants of the study will be interviewed during their following visits and asked to provide feedback on their experience as trial participants. If any areas of improvement are identified, we will discuss these with the TSC and TMG to further improve the trial design.

All participants will be asked about their willingness to participate if a sham procedure was included after randomisation and after surgery.

We will continue to engage with a wider group of national and international VLS patients on Facebook support groups through online questionnaires on the most suitable outcome measures for the definitive trial, sham procedure, the most appropriate platform to disseminate the results among a wider group of VLS patients and ways to increase the awareness of the study.

The PPI will form a keystone in the preparation and launch of the definitive trial.

Pre-submission

In rare cases where proposals do NOT involve patients and the public, clear justification must be provided.

8. Detailed Budget

Note: The SoECAT Form is included as a separate document.

Staff Costs

Budget Item 1

Staff Posts and Salaries

Staff name	Dr Sara Jasionowska

Staff type	Lead & Co-applicant

Organisation name	Royal Free London NHS Foundation Trust

Organisation type	NHS (UK)

Role	Plastic Surgery Core Trainee

Grade	Specialist Registars St1-2

Salary (£)
45,065

On-costs (£)
14,287

Current annual salary (£)	59,352.00

Annual Costs of Staff Posts

Rate (%) Please note, HEI staff and costs items are funded up to 80%
100

Base annual salary with rate	£59,352.00

	Year 1	Year 2	Year 3	
The year 1 value should always be 0%.	0	2	2	

Pre-submission

	Year 1	Year 2	Year 3	
FTE	10	10	10	

	Year 1	Year 2	Year 3	
Months	12	12	10	

	Year 1 (£)	Year 2 (£)	Year 3 (£)	Total (£)
Total rate calculation	£5,935.00	£6,054.00	£5,146.00	£17,135.00

Budget Item 2

Staff Posts and Salaries

Staff name	Dr Ben Langridge

Staff type	Lead & Co-applicant

Organisation name	Royal Free London NHS Foundation Trust

Organisation type	NHS (UK)

Role	NIHR Academic Clinical Fellow in Plastic and Reconstructive Surgery

Grade	Specialist Registars St3+

Salary (£)
56,768

On-costs (£)
17,585

Current annual salary (£)	74,353.00

Annual Costs of Staff Posts

Rate (%) Please note, HEI staff and costs items are funded up to 80%
100

Base annual salary with rate	£74,353.00

Pre-submission

	Year 1	Year 2	Year 3	
The year 1 value should always be 0%.	0	2	2	

	Year 1	Year 2	Year 3	
FTE	5	5	5	

	Year 1	Year 2	Year 3	
Months	12	12	10	

	Year 1 (£)	Year 2 (£)	Year 3 (£)	Total (£)
Total rate calculation	£3,718.00	£3,792.00	£3,223.00	£10,733.00

Budget Item 3

Staff Posts and Salaries

Staff name	Prof Peter Butler

Staff type	Lead & Co-applicant

Organisation name	Royal Free London NHS Foundation Trust

Organisation type	NHS (UK)

Role	Professor of Plastic and Reconstructive Surgery, Consultant Plastic and Reconstructive Surgeon

Grade	Consultant

Salary (£)
129,564

On-costs (£)
38,099

Current annual salary (£)	167,663.00

Annual Costs of Staff Posts

Rate (%) Please note, HEI staff and costs items are funded up to 80%
100

Pre-submission

Base annual salary with rate	£167,663.00

	Year 1	Year 2	Year 3	
The year 1 value should always be 0%.	0	2	2	

	Year 1	Year 2	Year 3	
FTE	10	10	10	

	Year 1	Year 2	Year 3	
Months	12	12	10	

	Year 1 (£)	Year 2 (£)	Year 3 (£)	Total (£)
Total rate calculation	£16,766.00	£17,102.00	£14,536.00	£48,404.00

Budget Item 4

Staff Posts and Salaries

Staff name	Dr Aurora Almadori

Staff type	Lead & Co-applicant

Organisation name	Royal Free London NHS Foundation Trust

Organisation type	NHS (UK)

Role	Consultant Plastic and Reconstructive Surgeon

Grade	Speciality Doctor

Salary (£)
53,896

On-costs (£)
16,775

Current annual salary (£)	70,671.00

Annual Costs of Staff Posts

Pre-submission

Rate (%) Please note, HEI staff and costs items are funded up to 80%
100

Base annual salary with rate	£70,671.00

	Year 1	Year 2	Year 3	
The year 1 value should always be 0%.	0	2	2	

	Year 1	Year 2	Year 3	
FTE	4	4	4	

	Year 1	Year 2	Year 3	
Months	12	12	10	

	Year 1 (£)	Year 2 (£)	Year 3 (£)	Total (£)
Total rate calculation	£2,827.00	£2,883.00	£2,451.00	£8,161.00

Budget Item 5

Staff Posts and Salaries

Staff name	Dr Esther Hansen

Staff type	Lead & Co-applicant

Organisation name	Royal Free London NHS Foundation Trust

Organisation type	NHS (UK)

Role	Consultant Clinical Psychologist

Grade	Band 8B

Salary (£)
70,037

On-costs (£)
28,744

Pre-submission

Current annual salary (£)	98,781.00

Annual Costs of Staff Posts

Rate (%) Please note, HEI staff and costs items are funded up to 80%
100

Base annual salary with rate	£98,781.00

	Year 1	Year 2	Year 3	
The year 1 value should always be 0%.	0	2	2	

	Year 1	Year 2	Year 3	
FTE	1	1	1	

	Year 1	Year 2	Year 3	
Months	12	12	10	

	Year 1 (£)	Year 2 (£)	Year 3 (£)	Total (£)
Total rate calculation	£988.00	£1,008.00	£856.00	£2,852.00

Budget Item 6

Staff Posts and Salaries

Staff name	Trial Nurse

Staff type	Research Staff

Organisation name	Royal Free London NHS Foundation Trust

Organisation type	NHS (UK)

Role	Trial Nurse

Grade	Band 7

Salary (£)
51,357

Pre-submission

On-costs (£)
23,404

Current annual salary (£)	74,761.00

Annual Costs of Staff Posts

Rate (%) Please note, HEI staff and costs items are funded up to 80%
100

Base annual salary with rate	£74,761.00

	Year 1	Year 2	Year 3	
The year 1 value should always be 0%.	0	2	2	

	Year 1	Year 2	Year 3	
FTE	20	20	20	

	Year 1	Year 2	Year 3	
Months	12	12	10	

	Year 1 (£)	Year 2 (£)	Year 3 (£)	Total (£)
Total rate calculation	£14,952.00	£15,251.00	£12,964.00	£43,167.00

Budget Item 7

Staff Posts and Salaries

Staff name	Prof Rosalind Simpson

Staff type	Lead & Co-applicant

Organisation name	University of Nottingham

Organisation type	HEI (UK)

Role	Consultant Dermatologist

Grade	Clinical Cons New A (NCB)

Pre-submission

Salary (£)
114,003

On-costs (£)
39,443

Current annual salary (£)	153,446.00

Annual Costs of Staff Posts

Rate (%) Please note, HEI staff and costs items are funded up to 80%
80

Base annual salary with rate	£122,756.80

	Year 1	Year 2	Year 3	
The year 1 value should always be 0%.	0	0	0	

	Year 1	Year 2	Year 3	
FTE	2.5	2.5	2.5	

	Year 1	Year 2	Year 3	
Months	12	12	10	

	Year 1 (£)	Year 2 (£)	Year 3 (£)	Total (£)
Total rate calculation	£3,069.00	£3,069.00	£2,557.00	£8,695.00

Budget Item 8

Staff Posts and Salaries

Staff name	Dr Dehbi Hakim-Moulay

Staff type	Lead & Co-applicant

Organisation name	University College London

Organisation type	HEI (UK)

Pre-submission

Role	Statistician

Grade	UCL Grade 9

Salary (£)
69,028

On-costs (£)
31,281

Current annual salary (£)	100,309.00

Annual Costs of Staff Posts

Rate (%) Please note, HEI staff and costs items are funded up to 80%
80

Base annual salary with rate	£80,247.20

	Year 1	Year 2	Year 3	
The year 1 value should always be 0%.	0	1	1	

	Year 1	Year 2	Year 3	
FTE	5	5	5	

	Year 1	Year 2	Year 3	
Months	12	12	10	

	Year 1 (£)	Year 2 (£)	Year 3 (£)	Total (£)
Total rate calculation	£4,012.00	£4,052.00	£3,411.00	£11,475.00

Budget Item 9

Staff Posts and Salaries

Staff name	Monica Panca

Staff type	Lead & Co-applicant

Pre-submission

Organisation name	University College London

Organisation type	HEI (UK)

Role	Healthcare Economist

Grade	UCL Grade 8

Salary (£)
52,946

On-costs (£)
24,390

Current annual salary (£)	77,336.00

Annual Costs of Staff Posts

Rate (%) Please note, HEI staff and costs items are funded up to 80%
80

Base annual salary with rate	£61,868.80

	Year 1	Year 2	Year 3	
The year 1 value should always be 0%.	0	2	2	

	Year 1	Year 2	Year 3	
FTE	5	5	5	

	Year 1	Year 2	Year 3	
Months	12	12	10	

	Year 1 (£)	Year 2 (£)	Year 3 (£)	Total (£)
Total rate calculation	£3,093.00	£3,155.00	£2,682.00	£8,930.00

Budget Item 10

Staff Posts and Salaries

Pre-submission

Staff name	Prof Norman Williams

Staff type	Lead & Co-applicant

Organisation name	University College London

Organisation type	HEI (UK)

Role	Clinical Trial Methodologist

Grade	UCL Grade 10

Salary (£)
77,495

On-costs (£)
32,680

Current annual salary (£)	110,175.00

Annual Costs of Staff Posts

Rate (%) Please note, HEI staff and costs items are funded up to 80%
80

Base annual salary with rate	£88,140.00

	Year 1	**Year 2**	**Year 3**	
The year 1 value should always be 0%.	0	0	0	

	Year 1	**Year 2**	**Year 3**	
FTE	5	5	5	

	Year 1	**Year 2**	**Year 3**	
Months	12	12	10	

	Year 1 (£)	**Year 2 (£)**	**Year 3 (£)**	**Total (£)**
Total rate calculation	£4,407.00	£4,407.00	£3,673.00	£12,487.00

Pre-submission

Budget Item 11

Staff Posts and Salaries

Staff name	Database Programmer

Staff type	Research Staff

Organisation name	University College London

Organisation type	HEI (UK)

Role	Database Programmer

Grade	UCL Grade 7

Salary (£)
40,521

On-costs (£)
20,086

Current annual salary (£)	60,607.00

Annual Costs of Staff Posts

Rate (%) Please note, HEI staff and costs items are funded up to 80%
80

Base annual salary with rate	£48,485.60

	Year 1	**Year 2**	**Year 3**	
The year 1 value should always be 0%.	0	0	0	

	Year 1	**Year 2**	**Year 3**	
FTE	10	0	0	

	Year 1	**Year 2**	**Year 3**	
Months	6	0	0	

Pre-submission

	Year 1 (£)	Year 2 (£)	Year 3 (£)	Total (£)
Total rate calculation	£2,424.00	£0.00	£0.00	£2,424.00

Budget Item 12

Staff Posts and Salaries

Staff name	R&D Support

Staff type	Other staff

Organisation name	Royal Free London NHS Foundation Trust

Organisation type	NHS (UK)

Role	Research and Development Departmental Support

Grade	Band 7

Salary (£)
51,357

On-costs (£)
23,404

Current annual salary (£)	74,761.00

Annual Costs of Staff Posts

Rate (%) Please note, HEI staff and costs items are funded up to 80%
100

Base annual salary with rate	£74,761.00

	Year 1	Year 2	Year 3	
The year 1 value should always be 0%.	0	1	1	

	Year 1	Year 2	Year 3	
FTE	10	5	5	

Pre-submission

	Year 1	Year 2	Year 3	
Months	12	12	10	

	Year 1 (£)	Year 2 (£)	Year 3 (£)	Total (£)
Total rate calculation	£7,476.00	£3,775.00	£3,178.00	£14,429.00

Budget Item 13

Staff Posts and Salaries

Staff name	Dr Victoria Swale

Staff type	Lead & Co-applicant

Organisation name	Royal Free London NHS Foundation Trust

Organisation type	NHS (UK)

Role	Consultant Dermatologist and Dermatopathologist

Grade	Consultant

Salary (£)
115,277

On-costs (£)
34,073

Current annual salary (£)	149,350.00

Annual Costs of Staff Posts

Rate (%) Please note, HEI staff and costs items are funded up to 80%
100

Base annual salary with rate	£149,350.00

	Year 1	Year 2	Year 3	
The year 1 value should always be 0%.	0	1	1	

Pre-submission

	Year 1	Year 2	Year 3	
FTE	1	1	1	

	Year 1	Year 2	Year 3	
Months	12	12	10	

	Year 1 (£)	Year 2 (£)	Year 3 (£)	Total (£)
Total rate calculation	£1,494.00	£1,508.00	£1,270.00	£4,272.00

Travel, Subsistence & Conference Costs

Budget Item 1

Item description	Monitoring Visits - Travel

Organisation Name	Royal Free London NHS Foundation Trust

Type of Cost	Travel

Organisation Type	NHS (UK)

Rate %	100

	Year 1	Year 2	Year 3	
Costs	£300.00	£300.00	£300.00	

	Year 1 (£)	Year 2 (£)	Year 3 (£)	Total (£)
Total rate calculation	£300.00	£300.00	£300.00	£900.00

Budget Item 2

Item description	Monitoring Visits - Subsistence

Organisation Name	Royal Free London NHS Foundation Trust

Type of Cost	Subsistence

Organisation Type	NHS (UK)

Rate %	100

Pre-submission

	Year 1	Year 2	Year 3	
Costs	£60.00	£60.00	£60.00	

	Year 1 (£)	Year 2 (£)	Year 3 (£)	Total (£)
Total rate calculation	£60.00	£60.00	£60.00	£180.00

Budget Item 3

Item description	Monitoring Visits - Overnight Stay

Organisation Name	Royal Free London NHS Foundation Trust

Type of Cost	Travel

Organisation Type	NHS (UK)

Rate %	100

	Year 1	Year 2	Year 3	
Costs	£300.00	£300.00	£300.00	

	Year 1 (£)	Year 2 (£)	Year 3 (£)	Total (£)
Total rate calculation	£300.00	£300.00	£300.00	£900.00

Budget Item 4

Item description	National Conferences - Fees

Organisation Name	Royal Free London NHS Foundation Trust

Type of Cost	Conference

Organisation Type	NHS (UK)

Rate %	100

	Year 1	Year 2	Year 3	

Pre-submission

Costs	£700.00	£700.00	£700.00	

	Year 1 (£)	Year 2 (£)	Year 3 (£)	Total (£)
Total rate calculation	£700.00	£700.00	£700.00	£2,100.00

Budget Item 5

Item description	National Conferences - Travel

Organisation Name	Royal Free London NHS Foundation Trust

Type of Cost	Travel

Organisation Type	NHS (UK)

Rate %	100

	Year 1	Year 2	Year 3	
Costs	£200.00	£200.00	£200.00	

	Year 1 (£)	Year 2 (£)	Year 3 (£)	Total (£)
Total rate calculation	£200.00	£200.00	£200.00	£600.00

Budget Item 6

Item description	National Conferences - Accomodation

Organisation Name	Royal Free London NHS Foundation Trust

Type of Cost	Travel

Organisation Type	NHS (UK)

Rate %	100

	Year 1	Year 2	Year 3	
Costs	£200.00	£200.00	£200.00	

Pre-submission

	Year 1 (£)	Year 2 (£)	Year 3 (£)	Total (£)
Total rate calculation	£200.00	£200.00	£200.00	£600.00

Budget Item 7

Item description	National Conferences - Subsistence

Organisation Name	Royal Free London NHS Foundation Trust

Type of Cost	Subsistence

Organisation Type	NHS (UK)

Rate %	100

	Year 1	**Year 2**	**Year 3**	
Costs	£200.00	£200.00	£200.00	

	Year 1 (£)	Year 2 (£)	Year 3 (£)	Total (£)
Total rate calculation	£200.00	£200.00	£200.00	£600.00

Budget Item 8

Item description	International Conference - Fees

Organisation Name	Royal Free London NHS Foundation Trust

Type of Cost	Travel

Organisation Type	NHS (UK)

Rate %	100

	Year 1	**Year 2**	**Year 3**	
Costs			£350.00	

	Year 1 (£)	Year 2 (£)	Year 3 (£)	Total (£)
Total rate calculation	£0.00	£0.00	£350.00	£350.00

Pre-submission

Budget Item 9

Item description	International Conference - Travel

Organisation Name	Royal Free London NHS Foundation Trust

Type of Cost	Travel

Organisation Type	NHS (UK)

Rate %	100

	Year 1	**Year 2**	**Year 3**	
Costs			£500.00	

	Year 1 (£)	**Year 2 (£)**	**Year 3 (£)**	**Total (£)**
Total rate calculation	£0.00	£0.00	£500.00	£500.00

Budget Item 10

Item description	International Conference - Accomodation

Organisation Name	Royal Free London NHS Foundation Trust

Type of Cost	Travel

Organisation Type	NHS (UK)

Rate %	100

	Year 1	**Year 2**	**Year 3**	
Costs			£300.00	

	Year 1 (£)	**Year 2 (£)**	**Year 3 (£)**	**Total (£)**
Total rate calculation	£0.00	£0.00	£300.00	£300.00

Budget Item 11

Item description	International Conference - Subsistence

Pre-submission

Organisation Name	Royal Free London NHS Foundation Trust

Type of Cost	Subsistence

Organisation Type	NHS (UK)

Rate %	100

	Year 1	**Year 2**	**Year 3**	
Costs			£300.00	

	Year 1 (£)	**Year 2 (£)**	**Year 3 (£)**	**Total (£)**
Total rate calculation	£0.00	£0.00	£300.00	£300.00

Equipment Costs

Budget Item 1

Item description	Phone for patient helpline

Organisation Name	Royal Free London NHS Foundation Trust

Organisation Type	NHS (UK)

Rate %	100

	Year 1	**Year 2**	**Year 3**	
Costs	£450.00			

	Year 1 (£)	**Year 2 (£)**	**Year 3 (£)**	**Total (£)**
Total rate calculation	£450.00	£0.00	£0.00	£450.00

Consumables

Budget Item 1

Item description	Printing and Stationery - Questionnaires

Organisation Name	Royal Free London NHS Foundation Trust

Pre-submission

Organisation Type	NHS (UK)

Rate %	100

	Year 1	**Year 2**	**Year 3**	
Costs	£550.00	£550.00	£550.00	

	Year 1 (£)	**Year 2 (£)**	**Year 3 (£)**	**Total (£)**
Total rate calculation	£550.00	£550.00	£550.00	£1,650.00

Patient & Public Involvement, Engagement and Participation Costs

Budget Item 1

Item description
Extended Patient Focus Group - Patient Reimbursement

Organisation Name	Royal Free London NHS Foundation Trust

Organisation Type	NHS (UK)

Rate %	100

	Year 1	**Year 2**	**Year 3**	
Costs	£600.00		£600.00	

	Year 1 (£)	**Year 2 (£)**	**Year 3 (£)**	**Total (£)**
Total rate calculation	£600.00	£0.00	£600.00	£1,200.00

Budget Item 2

Item description
PPI Representative Reimbursement - 36 Trial Management Meetings

Organisation Name	Royal Free London NHS Foundation Trust

Organisation Type	NHS (UK)

Rate %	100

Pre-submission

	Year 1	Year 2	Year 3	
Costs	£1,440.00	£1,440.00	£1,440.00	

	Year 1 (£)	Year 2 (£)	Year 3 (£)	Total (£)
Total rate calculation	£1,440.00	£1,440.00	£1,440.00	£4,320.00

Budget Item 3

Item description
PPI Representative Reimbursment - Trial Steering 4 Meetings

Organisation Name	Royal Free London NHS Foundation Trust

Organisation Type	NHS (UK)

Rate %	100

	Year 1	Year 2	Year 3	
Costs	£120.00	£120.00	£240.00	

	Year 1 (£)	Year 2 (£)	Year 3 (£)	Total (£)
Total rate calculation	£120.00	£120.00	£240.00	£480.00

Budget Item 4

Item description
Participant Event - Travel Reimbursement Nottingham - London

Organisation Name	Royal Free London NHS Foundation Trust

Organisation Type	NHS (UK)

Rate %	100

	Year 1	Year 2	Year 3	
Costs		£2,600.00	£1,950.00	

Pre-submission

	Year 1 (£)	Year 2 (£)	Year 3 (£)	Total (£)
Total rate calculation	£0.00	£2,600.00	£1,950.00	£4,550.00

Budget Item 5

Item description	Participant Event - Refreshments Provided

Organisation Name	Royal Free London NHS Foundation Trust

Organisation Type	NHS (UK)

Rate %	100

	Year 1	**Year 2**	**Year 3**	
Costs		£150.00	£150.00	

	Year 1 (£)	Year 2 (£)	Year 3 (£)	Total (£)
Total rate calculation	£0.00	£150.00	£150.00	£300.00

Other Direct Costs

Budget Item 1

Item description	Archiving Fee - 3 Boxes

Organisation Name	Royal Free London NHS Foundation Trust

Organisation Type	NHS (UK)

Rate %	100

	Year 1	**Year 2**	**Year 3**	
Costs			£1,542.00	

	Year 1 (£)	Year 2 (£)	Year 3 (£)	Total (£)
Total rate calculation	£0.00	£0.00	£1,542.00	£1,542.00

Pre-submission

Budget Item 2

Item description	Study Registration Fee

Organisation Name	Royal Free London NHS Foundation Trust

Organisation Type	NHS (UK)

Rate %	100

	Year 1	**Year 2**	**Year 3**	
Costs	£360.00			

	Year 1 (£)	**Year 2 (£)**	**Year 3 (£)**	**Total (£)**
Total rate calculation	£360.00	£0.00	£0.00	£360.00

Dissemination Costs

Indirect Costs

Budget Item 1

Item description	UCL Overhead

Organisation Name	University College London

Type of Cost	Estate Costs

Organisation Type	HEI (UK)

Rate % Please note for HEIs in the UK, up to 80% of FEC will be paid.
80

	Year 1	**Year 2**	**Year 3**	
Costs	£2,785.00	£2,193.00	£1,828.00	

	Year 1 (£)	**Year 2 (£)**	**Year 3 (£)**	**Total (£)**
Total rate calculation	£2,228.00	£1,754.00	£1,462.00	£5,444.00

Pre-submission

Budget Item 2

Item description	UCL Overhead

Organisation Name	University College London

Type of Cost	Other Indirect Costs

Organisation Type	HEI (UK)

Rate % Please note for HEIs in the UK, up to 80% of FEC will be paid.
80

	Year 1	**Year 2**	**Year 3**	
Costs	£12,150.00	£9,112.00	£7,594.00	

	Year 1 (£)	**Year 2 (£)**	**Year 3 (£)**	**Total (£)**
Total rate calculation	£9,720.00	£7,290.00	£6,075.00	£23,085.00

Budget Item 3

Item description	UON Overhead

Organisation Name	University of Nottingham

Type of Cost	Estate Costs

Organisation Type	HEI (UK)

Rate % Please note for HEIs in the UK, up to 80% of FEC will be paid.
80

	Year 1	**Year 2**	**Year 3**	
Costs	£499.62	£499.62	£416.35	

	Year 1 (£)	**Year 2 (£)**	**Year 3 (£)**	**Total (£)**
Total rate calculation	£400.00	£400.00	£333.00	£1,133.00

Pre-submission

Budget Item 4

Item description	UON Overhead

Organisation Name	University of Nottingham

Type of Cost	Other Indirect Costs

Organisation Type	HEI (UK)

Rate % Please note for HEIs in the UK, up to 80% of FEC will be paid.
80

	Year 1	**Year 2**	**Year 3**	
Costs	£1,573.43	£1,573.43	£1,311.19	

	Year 1 (£)	**Year 2 (£)**	**Year 3 (£)**	**Total (£)**
Total rate calculation	£1,259.00	£1,259.00	£1,049.00	£3,567.00

Justification of costs
Monitoring Visits: 3x tickets at £100 per annum: Necessary for project monitoring and ensuring the effective implementation of activities. These visits facilitate real-time observation, feedback, and collaboration. 3x £20 subsistence per visit, per annum: Covers essential expenses related to meals and incidentals during monitoring visits, ensuring the well-being of staff and the success of visits. 3x overnight hotel stays per annum: Required for monitoring visits that extend beyond a single day, ensuring sufficient time for comprehensive assessments. National Conferences: Conference fees for 2 participants per annum £350: Supports professional development, knowledge sharing, and networking opportunities crucial for project success. National travel ticket £100 x 2 conference participants per annum: Covers travel expenses for participation, promoting collaboration and dissemination of project findings. 2 x 2 overnight stays per conference participant per annum: Ensures participants can fully engage in conference activities by accommodating overnight stays when necessary. £100 per day, for 2 days, per conference participant, per annum: Covers additional expenses associated with attending conferences, such as meals and incidental costs. International Conference: 2 attendees for international conference: Enables global exposure, fostering cross-cultural collaborations and knowledge exchange. 2 attendees for international conference flights: Essential for participation in international forums, broadening the project's impact and visibility. 2 attendees for international conference accommodation (2 days): Ensures the comfort and safety of attendees during international events. 2 attendees for international conference subsistence (2 days): Covers essential daily expenses for participants during international conferences. Patient Involvement: 30x patient reimbursement @ £20: Provides fair compensation to patients for their time and effort, incentivizing their continued participation in the project from recruitment to completion. Patient Representative Meetings:

Pre-submission

£120 x 36 meetings: Compensates patient representatives for their valuable time and insights, promoting sustained engagement and commitment to the project.
1 per year, 2 in the last year: Supports ongoing collaboration and ensures representation of patient perspectives throughout the project's duration.
Allocating £65 per day return ticket for 10 participants (Nottingham to London) fosters in-person collaboration in hybrid meetings, enhancing engagement. A £15 provision per in-person participant ensures a conducive environment with tea and coffee, supporting a comfortable and collaborative atmosphere for those physically present.
Allocating £450 for a dedicated patient helpline ensures effective communication.
Three archiving boxes at £519 each support secure document storage.
£550 per study year for printing and stationery guarantees the production of accurate study materials.
A standard study registration fee of £360 covers essential administrative costs, ensuring compliance and bolstering the study's credibility.
Study team costs:
Chief investigator cost (10% FTE)
Clinical Trial Methodologist (5% FTE)
Statistician (5% FTE)
Healthcare Economist (5% FTE)
Consultant Dermatologists at Nottingham (2.5% FTE) and at Royal Free (1% FTE)
Consultant Clinical Psychologist (1% FTE)
Consultant Plastic Surgeon (4% FTE)
Clinical Research Fellows (10% FTE and 5% FTE)

Totals

	Year 1 (£)	Year 2 (£)	Year 3 (£)	Total (£)
Staff Costs	£71,161.00	£66,056.00	£55,947.00	£193,164.00
Travel, Subsistence & Conference Costs	£1,960.00	£1,960.00	£3,410.00	£7,330.00
Equipment Costs	£450.00	£0.00	£0.00	£450.00
Consumables	£550.00	£550.00	£550.00	£1,650.00
PPIEP Costs	£2,160.00	£4,310.00	£4,380.00	£10,850.00
Other Direct Costs	£360.00	£0.00	£1,542.00	£1,902.00
Dissemination Costs	£0.00	£0.00	£0.00	£0.00
Indirect Costs	£13,607.00	£10,703.00	£8,919.00	£33,229.00
Total	**£90,248.00**	**£83,579.00**	**£74,748.00**	**£248,575.00**

9. Management & Governance

Is Clinical Trials Authorisation required?	No

Does your project require ethics approval?	Yes

If yes, has ethics approval already been obtained?	No

10. Uploads

Pre-submission

MHRA Regulatory Meeting Minutes

Use of lipotransfer/free fat grafts for facial reconstruction in Systemic Sclerosis patients

MEETING	**MHRA / UCL –** Use of lipotransfer/free fat grafts for facial reconstruction in Systemic Sclerosis patients

DATE/TIME/SITE	23rd June 2016

PURPOSE	To seek advice from the MHRA on the classification of lipotransfer /fat grafting as an intervention and to ascertain what regulatory requirements might be needed.

INVITED	Name	Attended
PB	Professor Peter Butler (UCL Professor of Plastic and Reconstructive Surgery)	X
CR	Caroline Ryan Ph.D. (Research Coordinator, UCL Division of Surgery and Interventional Science)	X
MG	Michelle Griffin (Clinical Research Fellow, UCL Division of Surgery and Interventional Science)	X
SE	Simon Eaglestone Ph.D. (Translational Research Manager, UCL)	X
PT	Pamela Tranter Ph.D. (Interim Principal Translational Research Manager, UCL)	X
MA	Martin Anyim Ph.D. (Translational Research Manager, UCL)	X
KW	Dr Kirsty Wydenbach - Medical Assessor	X
GMc	Dr Graham McNaughton -Pharmaceutical Assessor	X
DJ	Mr David Jones - Non-Clinical Assessor	X

AGENDA	Item	Led by
1.	Presentation on the clinical experience in facial reconstruction using lipotransfer /free fat graft in systemic sclerosis patients	PB
2.	Discussion driven by the presentation and the 3 questions set-out in the briefing document	All

MINUTES
1. Presentation on the clinical experience in facial reconstruction using lipotransfer /free fat graft in systemic sclerosis patients
PB set out the clinical experience of using autologous lipotransfer for facial Systemic Sclerosis (SSc) at the Royal Free Hospital. This technique has been used to reconstruct patients since 1999. Lipotransfer has been used in patients with facial lipoatrophy and microstomia as a consequence of SSc since 2002. The use of free fat transfer to reconstruct the facial tissues resulted in an improved and more predictable surgical outcome in comparison with flaps and grafts. Over the years, SSc patients reported very satisfactory outcomes with this treatment option. The patients reported that their microstomia and oral continence was improved, as well as an unexpected finding of a softening of fibrotic facial tissues. A retrospective evaluation of this patient series revealed that autologous lipotransfer positively impacts the effects of fibrosis in facial SSc. (Refer to presentation)
2. Discussion around the presentation and 3 questions set-out in the briefing document

1) We would classify lipotransfer/ fat grafting as an Intervention. Does the MHRA agree?

DJ – Stated that what was being proposed was not a medicinal product at the moment.

PB – In agreement with DJ, but stated that this was the ultimate aim.

GMc – In order for this to be a medicinal product, it would depend on the manufacturing processes involved and being able to use the product for the same essential function as outlined in the presentation. At present it was not considered a medicinal product.

2) We aim to determine the effector cell/mechanism of action of the lipoaspirate. Through mechanistic studies we hope to advance to potential improvements in the intervention, potential drug design and molecule development. Secondary to this we may wish to perform cell enrichment at a future point to refine our technique and enhance our clinical outcome. What does the MHRA need to be in place to advance to this step?

PB – Centrifugation of the fat is the main process used at present to separate oil and blood from the fat graft. This is then re-injected into the patient in the area requiring reconstruction.

GMc – Indicated that he thought that the fat isolation through centrifugation did not result in a medicinal product. If there was any expansion of the cells then it potentially could become a medicinal product. The MHRA has a clinical trials group who could advise on whether this is a medicinal product. There is also the Borderlines Group.

DJ – Stated that he could provide a link to the necessary form if required.

GMc – The European Medicines Agency provide a similar service. They would, however, require variables.

PB – Stated that in some previous work an enzymatic digestion step in the theatre was considered a significant manipulation by the FDA. The selection of cells and their washing was not considered a significant manipulation. In the current clinical process, centrifugation is used as a gravity enrichment for the cells.

GMc – Did not consider the centrifugation and/or washing of lipoaspirate to be a significant manipulation. However any enzymatic digestion is likely to be considered a significant manipulation by the MHRA too. This would need to be performed in a GMP/GLP facility.

3) There are many processing techniques that could enhance engraftment and efficacy. Some will take place in the operating room, such as washing or enzymatic digestion, but some may need to take place within a laboratory setting i.e. processing outside the operating room in a licensed and accredited laboratory. What extent of processing for this particular intervention is acceptable with the MHRA?

GMc – Was of the opinion that the cells would need to be characterised and linked back to the clinical effect.

DJ – In agreement with GMc, but believed that this was some way off at this stage.

MHRA email chain on the use of lipotrasfer

From: **BUTLER, Peter (ROYAL FREE LONDON NHS FOUNDATION TRUST)** peter.butler1@nhs.net
Subject: Fwd: Autologous fat processing
Date: 26 October 2023 at 17:45
To: JASIONOWSKA, Sara (ROYAL FREE LONDON NHS FOUNDATION TRUST) s.jasionowska@nhs.net

PB

Begin forwarded message:

From: "OKOJIE, Gloria (ROYAL FREE LONDON NHS FOUNDATION TRUST)" <g.okojie@nhs.net>
Subject: **RE: Autologous fat processing**
Date: 7 March 2017 at 14:10:28 GMT
To: "Tranter, Pamela" <p.tranter@ucl.ac.uk>
Cc: "Fuller, Barry" <b.fuller@ucl.ac.uk>, "BUTLER, Peter (ROYAL FREE LONDON NHS FOUNDATION TRUST)" <peter.butler1@nhs.net>, "Ryan, Caroline" <caroline.ryan@ucl.ac.uk>

Dear Pamela,

Thank you for this – much appreciated!

We will follow up with our internal governance, the minutes feeds well into it.

BW,
Gloria

From: Tranter, Pamela [mailto:p.tranter@ucl.ac.uk]
Sent: 14 February 2017 09:51
To: OKOJIE, Gloria (ROYAL FREE LONDON NHS FOUNDATION TRUST)
Cc: Fuller, Barry; BUTLER, Peter (ROYAL FREE LONDON NHS FOUNDATION TRUST); Ryan, Caroline
Subject: RE: Autologous fat processing

Dear Gloria

Please final attached the minutes that we took from our discussion with the MHRA which was an advisory meeting.

Best wishes

Pamela

From: Partington, Leanne
Sent: 06 February 2017 15:31
To: OKOJIE, Gloria (ROYAL FREE LONDON NHS FOUNDATION TRUST); Tranter, Pamela
Cc: Fuller, Barry
Subject: RE: Autologous fat processing

Dear Gloria,

I had a quick chat with Pamela Tranter, the Head of the TRG, regarding your call.
She was involved in the discussion with the regulators and is probably your best point of contact for this matter.
I have copied her into this mail.

Kindest regards,
Leanne.

Dr Leanne Partington
Translational Research Manager | Translational Research Group
Translational Research Office | School of Life and Medical Sciences | University College London | Maple House, 1st Floor 1A | 149 Tottenham Court Road | London | W1T 7JA
Tel: +44 (0) 20 7679 0751 (Internal: 40751) | email: l.partington@ucl.ac.uk) | Web:

Pre-submission

Tel: +44 (0) 20 7679 0751 (Internal: 40751) | email: l.partington@ucl.ac.uk) | Web: www.ucl.ac.uk/translational-research

---

From: OKOJIE, Gloria (ROYAL FREE LONDON NHS FOUNDATION TRUST) [mailto:g.okojie@nhs.net]
Sent: 06 February 2017 15:26
To: Partington, Leanne <l.partington@ucl.ac.uk>
Cc: Fuller, Barry <b.fuller@ucl.ac.uk>
Subject: Autologous fat processing

Dear Leanne,

Thank you for talking to me earlier – much appreciated.

We are trying to establish any paperwork that may be available for this pathway. We understand from Prof. Peter Butler's team that the process was discussed with MHRA via your office and this was not considered a cell therapy or ATMP but a sizing procedure.

Any information you have on this would be really appreciated.

BW,
Gloria

Gloria Okojie
Head of Quality & Licensing – Human Tissue & Organ
BCF Integration – Quality Governance Lead
Endoscopy Service – Quality Governance Lead
Royal Free London NHS Foundation Trust
Royal Free Hospital
Transplantation & Specialist Services
Yellow Hut
69 Fleet Road
London NW3 2QG

☏ *0207 794 0500 (Ext. 34828)*
g.okojie@nhs.net

Websites: www.hta.gov.uk
www.nhsbt.nhs.uk
http://freenet/freenetcms/Default.aspx?&s=38&p=1257&m=1786

Please consider the environment before printing this e-mail.

world class expertise local care

Pre-submission

****
****

This message may contain confidential information. If you are not the

Please do not disclose, copy or distribute information in this e-mail

to do so is strictly prohibited and may be unlawful.

MHRA email chain on Platel-rich Plasma

From: "BUTLER, Peter (ROYAL FREE LONDON NHS FOUNDATION TRUST)" <peter.butler1@nhs.net>
Subject: Fwd: CEC 151171: Is a CTA needed?
Date: 30 October 2023 at 10:50:55 GMT
To: "JASIONOWSKA, Sara (ROYAL FREE LONDON NHS FOUNDATION TRUST)" <s.jasionowska@nhs.net>, "LANGRIDGE, Benjamin (ROYAL FREE LONDON NHS FOUNDATION TRUST)" <b.langridge1@nhs.net>

Begin forwarded message:
From: David Jones <Davidrjones@hotmail.co.uk>
Subject: FW: CEC 151171: Is a CTA needed?
Date: 27 October 2023 at 11:16:16 BST
To: "Cooksley, Helen" <h.cooksley@ucl.ac.uk>, "BUTLER, Peter (ROYAL FREE LONDON NHS FOUNDATION TRUST)" <peter.butler1@nhs.net>

You don't often get email from davidrjones@hotmail.co.uk. Learn why this is important

This message originated from outside of NHSmail. Please do not click links or open attachments unless you recognise the sender and know the content is safe.
MHRA resent their response

David

David R Jones FRSB FBTS
Expert Pharmaco-Toxicologist

This message and any attachments contain information intended solely for the addressee which is confidential or private in nature and subject to legal privilege. If you receive this message in error please notify the sender immediately by e-mail and thereafter delete the message and do not disclose the content in any way.

From: Clinical Trial Helpline <ctdhelpline@mhra.gov.uk>
Sent: Friday, October 27, 2023 10:59 AM
To: David Jones <davidrjones@hotmail.co.uk>
Subject: CEC 151171: Is a CTA needed?

Dear David,

We apologise for the delay in replying to your enquiry.

The critical element to decide if the proposed trial fulfils the definition of a clinical

trial of an investigational medicinal product (CTIMP), as defined by the EU Directive 2001/20/EC is whether PRP is an IMP or not and whether there is assessment of clinical outcomes or not. If the PRP is determined to be an IMP and the study endpoints are clinical (which has not been clarified in your enquiry but is assumed for this consideration) then this study would be regarded as a CTIMP. Therefore, both aspects need to be considered by the sponsor of the trial to make a final decision.

If a PRP product has been subject to additional manipulation (for example, addition of calcium chloride) it would fall within the definition of a medicinal product whereas a product that has not been subject to additional manipulation steps falls outside the definition. If you require further assistance on this aspect, you can consider discussing your enquiry with MHRA borderline. Ed Scully (Borderline Lead) has reviewed your enquiry and is willing to provide further support in case of need: Ed.Scully@mhra.gov.uk

- Human plasma

CTIMP	Not CTIMP
Platelet rich plasma (PRP) where the process involves mixing the PRP with activators, such as ascorbic acid and thrombin, to produce a gel. This is considered as manipulation and therefore the resulting product is a medicine. The gel is proposed as having a pharmacological action and is used to treat diabetic foot ulcers.	Autologous platelet rich plasma, collected, processed and administered all within a single surgical procedure with no other substances used for activation or manipulation and used in a dental procedure.
Unmanipulated PRP vs steroid in a study evaluating efficacy in impingement syndrome of the shoulder. Although the PRP is not a medicine the study is evaluating steroid as a comparator, and as that is a medicine the study has to be a CTIMP (providing criteria in columns C, D and E are also met).	Autologous blood drawn into an anticoagulant (such as sodium citrate or EDTA), centrifuged and buffered to achieve physiological pH (for example with bicarbonate), resulting in platelet rich plasma. The anticoagulant and buffering are not manipulating the platelets and so the product is not considered a medicine.
Platelet rich growth factors used in dentistry that are subject to additional manipulation, including addition of calcium chloride, fall within the definition of a medicinal product whereas a product that has not been subject to these additional manipulation steps falls outside the definition of a medicinal product. If in doubt then contact the MHRA Borderline team.	

More information on 'When a clinical trial authorisation (CTA) is needed' can be found on the MHRA website: https://www.gov.uk/guidance/clinical-trials-for-medicines-apply-for-authorisation-in-the-uk#when-a-clinical-trial-authorisation-cta-is-needed
You are reminded that From October 2021 the 'SCOPE' advice service will only

be available via self-service using the guidance on this webpage.

Leonard
MHRA Customer Experience Centre
Clinical Investigations Trials
Medicines and Healthcare products Regulatory Agency
10 South Colonnade, Canary Wharf, London E14 4PU
Telephone 020 3080 6456
Email: ctdhelpline@mhra.gov.uk

gov.uk/mhra
Stay connected

Medicines & Healthcare products
Regulatory Agency

Read our guidance on coronavirus (COVID-19)

For information on how the Agency uses your personal data and your data protection rights, please see our three centres' Privacy Notices: MHRA, CPRD and NIBSC.

From: Clinical Trial Helpline <ctdhelpline@mhra.gov.uk>
Sent: Tuesday, June 13, 2023 9:27 AM
To: David Jones <davidrjones@hotmail.co.uk>
Subject: CEC 151171: Is a CTA needed?

Dear David

Thank you for your email dated 9 June 2023 re Is a CTA needed?

We have redirected your query to our experts for further input.

If you have not heard from us within 14 calendar days, please contact us.

Our opening hours are Mon – Fri 08:30am to 4:30pm (excluding UK Public Holidays)

Kind regards

Leonard

MHRA Customer Experience Centre
Clinical Investigations Trials
Medicines and Healthcare products Regulatory Agency
10 South Colonnade, Canary Wharf, London E14 4PU
Direct Line: 020 3080 6456
ctdhelpline@mhra.gov.uk
gov.uk/mhra
Stay connected

Read our guidance on coronavirus (COVID-19)

For information on how the Agency uses your personal data and your data protection rights, please see our three centres' Privacy Notices: MHRA, CPRD and NIBSC.

Medicines & Healthcare products
Regulatory Agency

From: David Jones <Davidrjones@hotmail.co.uk>
Sent: 09 June 2023 11:23
To: Clinical Trial Helpline <ctdhelpline@mhra.gov.uk>
Subject: Is a CTA needed?

Vulval lichen sclerosus (VLS) is a chronic and progressive condition of unknown aetiology which predominantly affects the anogenital region. A hospital group are currently planning a clinical trial involving the use of Platelet-Rich Plasma (PRP) as an intervention for patients with VLS. The study will be designed as a randomised, controlled, feasibility study. The 3 arms will be control (standard of care) vs PRP vs Autologous Fat Grafting (AFG). The two interventions, PRP and AFG, are surgical procedures that are widely practiced internationally, including the UK, for VLS but their evidence base is poor, hence the desire to generate good data.

PRP is currently used for many wider indications for example in Plastic surgery, Orthopaedic surgery and Dermatology.

The investigators will prepare and administer PRP in the standard fractionation manner. A small autologous blood sample will be drawn into an anticoagulant-lined blood bottle, centrifuged, and the platelet rich fraction will be immediately injected into the affected vulvar area without any modification, activation, or addition of products

The question is, does this constitute a clinical trial that requires approval from MHRA?

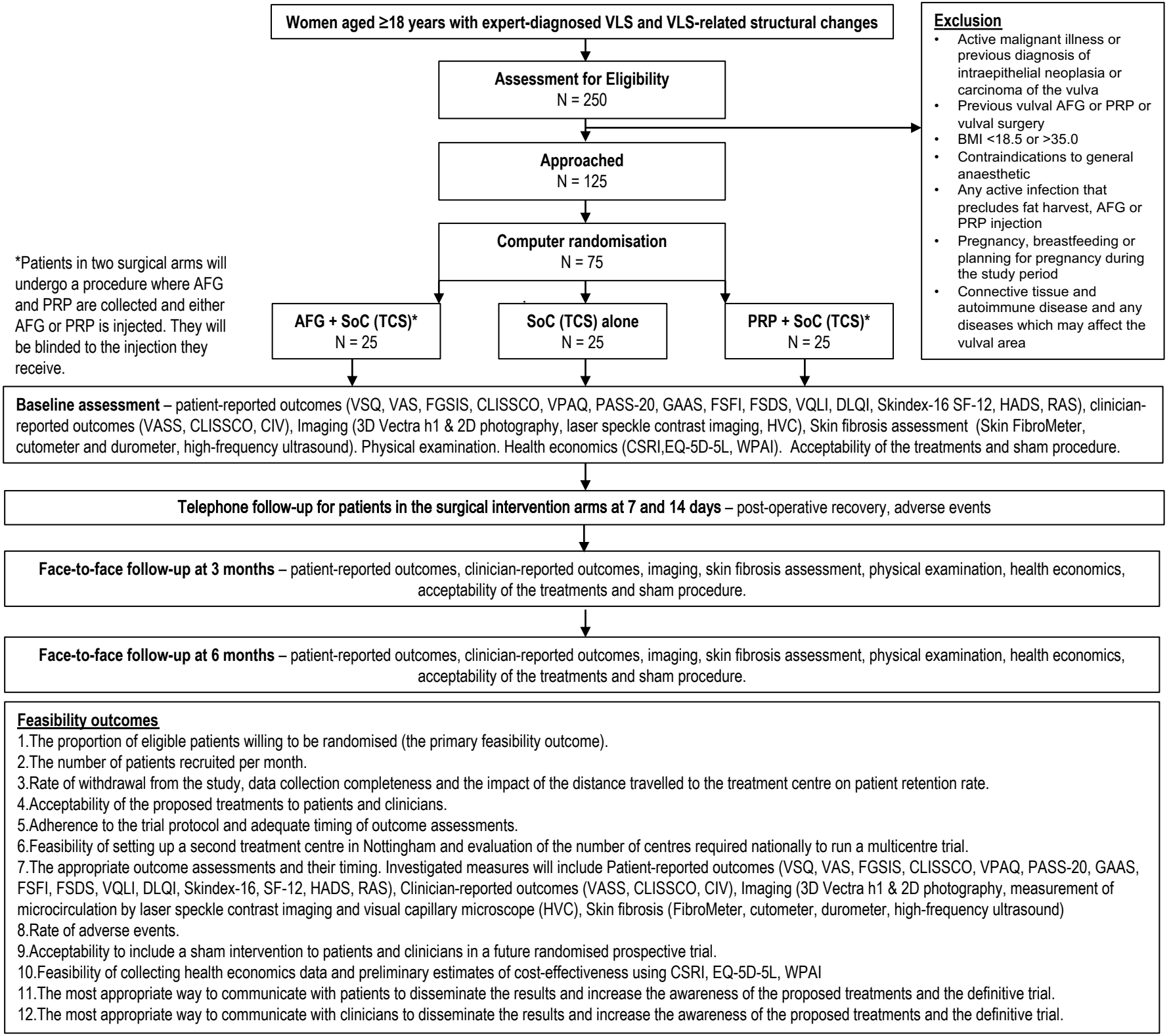
Thank you

David

David R Jones FRSB FBTS
Expert Pharmaco-Toxicologist

This message and any attachments contain information intended solely for the addressee which is confidential or private in nature and subject to legal privilege. If you receive this message in error please notify the sender immediately by e-mail and thereafter delete the message and do not disclose the content in any way.

Pre-submission


Women aged ≥18 years with expert-diagnosed VLS and VLS-related structural changes
Assessment for Eligibility
N = 250
Approached
N = 125
Computer randomisation
N = 75
AFG + SoC (TCS)*
N = 25
SoC (TCS) alone
N = 25
PRP + SoC (TCS)*
N = 25
Exclusion
• Active malignant illness or previous diagnosis of intraepithelial neoplasia or carcinoma of the vulva
• Previous vulval AFG or PRP or vulval surgery
• BMI <18.5 or >35.0
• Contraindications to general anaesthetic
• Any active infection that precludes fat harvest, AFG or PRP injection
• Pregnancy, breastfeeding or planning for pregnancy during the study period
• Connective tissue and autoimmune disease and any diseases which may affect the vulval area
*Patients in two surgical arms will undergo a procedure where AFG and PRP are collected and either AFG or PRP is injected. They will be blinded to the injection they receive.
Baseline assessment – patient-reported outcomes (VSQ, VAS, FGSIS, CLISSCO, VPAQ, PASS-20, GAAS, FSFI, FSDS, VQLI, DLQI, Skindex-16 SF-12, HADS, RAS), clinician-reported outcomes (VASS, CLISSCO, CIV), Imaging (3D Vectra h1 & 2D photography, laser speckle contrast imaging, HVC), Skin fibrosis assessment (Skin FibroMeter, cutometer and durometer, high-frequency ultrasound). Physical examination. Health economics (CSRI,EQ-5D-5L, WPAI). Acceptability of the treatments and sham procedure.
Telephone follow-up for patients in the surgical intervention arms at 7 and 14 days – post-operative recovery, adverse events
Face-to-face follow-up at 3 months – patient-reported outcomes, clinician-reported outcomes, imaging, skin fibrosis assessment, physical examination, health economics, acceptability of the treatments and sham procedure.
Face-to-face follow-up at 6 months – patient-reported outcomes, clinician-reported outcomes, imaging, skin fibrosis assessment, physical examination, health economics, acceptability of the treatments and sham procedure.
Feasibility outcomes
1.The proportion of eligible patients willing to be randomised (the primary feasibility outcome).
2.The number of patients recruited per month.
3.Rate of withdrawal from the study, data collection completeness and the impact of the distance travelled to the treatment centre on patient retention rate.
4.Acceptability of the proposed treatments to patients and clinicians.
5.Adherence to the trial protocol and adequate timing of outcome assessments.
6.Feasibility of setting up a second treatment centre in Nottingham and evaluation of the number of centres required nationally to run a multicentre trial.
7.The appropriate outcome assessments and their timing. Investigated measures will include Patient-reported outcomes (VSQ, VAS, FGSIS, CLISSCO, VPAQ, PASS-20, GAAS, FSFI, FSDS, VQLI, DLQI, Skindex-16, SF-12, HADS, RAS), Clinician-reported outcomes (VASS, CLISSCO, CIV), Imaging (3D Vectra h1 & 2D photography, measurement of microcirculation by laser speckle contrast imaging and visual capillary microscope (HVC), Skin fibrosis (FibroMeter, cutometer, durometer, high-frequency ultrasound)
8.Rate of adverse events.
9.Acceptability to include a sham intervention to patients and clinicians in a future randomised prospective trial.
10.Feasibility of collecting health economics data and preliminary estimates of cost-effectiveness using CSRI, EQ-5D-5L, WPAI
11.The most appropriate way to communicate with patients to disseminate the results and increase the awareness of the proposed treatments and the definitive trial.
12.The most appropriate way to communicate with clinicians to disseminate the results and increase the awareness of the proposed treatments and the definitive trial.

References

1. Corazza M, Schettini N, Zedde P, Borghi A. Vulvar Lichen Sclerosus from Pathophysiology to Therapeutic Approaches: Evidence and Prospects. Biomedicines. 2021 Aug 3;9(8).
2. Goldstein AT, Marinoff SC, Christopher K, Srodon M. Prevalence of vulvar lichen sclerosus in a general gynecology practice. J Reprod Med. 2005 Jul;50(7):477–80.
3. Leibovitz A, Kaplun V, Saposhnicov N, Habot B. Vulvovaginal examinations in elderly nursing home women residents. Arch Gerontol Geriatr [Internet]. 2000 Aug [cited 2023 Nov 19];31(1):1–4. Available from: https://pubmed.ncbi.nlm.nih.gov/10989157/
4. Cooper SM, Gao XH, Powell JJ, Wojnarowska F. Does Treatment of Vulvar Lichen Sclerosus Influence Its Prognosis? Arch Dermatol. 2004 Jun 1;140(6).
5. Goldstein AT, Mitchell L, Govind V, Heller D. A randomized double-blind placebo-controlled trial of autologous platelet-rich plasma intradermal injections for the treatment of vulvar lichen sclerosus. J Am Acad Dermatol [Internet]. 2019 Jun 1 [cited 2023 Oct 31];80(6):1788–9. Available from: https://pubmed.ncbi.nlm.nih.gov/30639885/
6. Swisher E. Lichen sclerosus: Review of a vulvar dystrophy with malignant potential. Contemp Ob Gyn. 2021 Aug 9;66(8).
7. Orszulak D, Dulska A, Niziński K, Skowronek K, Bodziony J, Stojko R, et al. Pediatric Vulvar Lichen Sclerosus-A Review of the Literature. Int J Environ Res Public Health. 2021 Jul 4;18(13).
8. Chin S, Scurry J, Bradford J, Lee G, Fischer G. Association of Topical Corticosteroids With Reduced Vulvar Squamous Cell Carcinoma Recurrence in Patients With Vulvar Lichen Sclerosus. JAMA Dermatol. 2020 Jul 1;156(7):813.
9. Krapf JM, Mitchell L, Holton MA, Goldstein AT. Vulvar Lichen Sclerosus: Current Perspectives Int J Womens Health. 2020 Jan;Volume 12:11–20.
10. Singh N, Ghatage P. Etiology, Clinical Features, and Diagnosis of Vulvar Lichen Sclerosus: A Scoping Review. Obstet Gynecol Int. 2020 Apr 21;2020:1–8.
11. Renaud-Vilmer C, Cavelier-Balloy B, Porcher R, Dubertret L. Vulvar Lichen Sclerosus. Arch Dermatol. 2004 Jun 1;140(6).
12. Lee A, Bradford J, Fischer G. Long-term Management of Adult Vulvar Lichen Sclerosus. JAMA Dermatol. 2015 Oct 1;151(10):1061.
13. Pergialiotis V, Bellos I, Biliou EC, Varnava P, Mitsopoulou D, Doumouchtsis SK. An arm-based network meta-analysis on treatments for vulvar lichen sclerosus and a call for development of core outcome sets. Am J Obstet Gynecol. 2020 Jun;222(6):542-550.e6.
14. Smith AB, Muhammad NI, Cigna ST, Krapf JM. A systematic review of sexual health consequences among women with lichen sclerosus. Sex Med Rev. 2023 Jan 19;11(1):8–14.
15. Union PO of the E. Scoping study on evidence to tackle high-burden under-researched medical conditions : discussion paper. 2023 Jan 17 [cited 2023 Oct 31]; Available from: https://op.europa.eu/en/publication-detail/-/publication/eae32303-96e3-11ed-b508-01aa75ed71a1/language-en/format-PDF/source-278963958
16. Almadori A, Hansen E, Boyle D, Zenner N, Swale V, Reid W, et al. Fat Grafting Improves Fibrosis and Scarring in Vulvar Lichen Sclerosus: Results From a Prospective Cohort Study. J Low Genit Tract Dis [Internet]. 2020 Jul 1 [cited 2023 Oct 31];24(3):305–10. Available from: https://pubmed.ncbi.nlm.nih.gov/32205767/
17. British Association of Dermatologists [Internet]. [cited 2023 Oct 31]. Available from: https://www.bad.org.uk/one-in-five-women-with-a-vulval-health-condition-contemplate-self-harm-or-suicide/
18. Simpson RC, Cooper SM, Kirtschig G, Larsen S, Lawton S, McPhee M, et al. Future research priorities for lichen sclerosus – results of a James Lind Alliance Priority Setting Partnership. Br J Dermatol [Internet]. 2019 May 1 [cited 2023 Oct 31];180(5):1236. Available from: /pmc/articles/PMC6850137/
19. Gutierrez-Ontalvilla P, Giner F, Vidal L, Iborra M. The Effect of Lipofilling and Platelet-Rich Plasma on Patients with Moderate-Severe Vulvar Lichen Sclerosus who were Non-Responders to Topical Clobetasol Propionate: A Randomized Pilot Study. Aesthetic Plast Surg [Internet]. 2022 Oct 1 [cited 2023 Oct 31];46(5):2469–79. Available from: https://pubmed.ncbi.nlm.nih.gov/35048150/
20. Arthurs JR. A Study to Evaluate Platelet Rich Plasma to Treat Vulvar Lichen Sclerosus | ClinicalTrials.gov [Internet]. [cited 2023 Oct 31]. Available from: https://clinicaltrials.gov/study/NCT04967170?cond=NCT04967170&rank=1
21. Fundacion Eduardo Anitua. Plasma Rich in Growth Factors (PRGF) for the Treatment of Lichen Sclerosus Atrophicus of the Vulva | ClinicalTrials.gov [Internet]. [cited 2023 Oct 31]. Available from: https://clinicaltrials.gov/study/NCT05364515
22. Almadori A, Zenner N, Boyle D, Swale V, Reid W, Maclean A, et al. Development and Validation of a Clinical Grading Scale to Assess the Vulvar Region: The Vulvar Architecture Severity Scale. Aesthet Surg J. 2020 Nov 19;40(12):1319–26.
23. Almadori A, Speiser S, Ashby I, Lacher R, Bishop D, Mosahebi A, et al. Portable three-dimensional imaging to monitor small volume enhancement in face, vulva, and hand: A comparative study. Journal of Plastic, Reconstructive & Aesthetic Surgery. 2022 Sep;75(9):3574–85.
24. Staniszewska S, Brett J, Simera I, Seers K, Mockford C, Goodlad S, et al. GRIPP2 reporting checklists: tools to improve reporting of patient and public involvement in research. BMJ. 2017 Aug 2;j3453.
25. Goodrum CA, Leighton PA, Simpson RC. Outcome domains in lichen sclerosus. Br J Dermatol [Internet]. 2020 Nov 1 [cited 2023 Oct 31];183(5):966–8. Available from: https://pubmed.ncbi.nlm.nih.gov/32471015/
26. NIHR. Improving inclusion of under-served groups in clinical research: Guidance from the NIHR-INCLUDE project. . 2020.
27. Simpson RC, Kirtschig G, Selk A, von Seitzberg S, Vittrup G, Bissonnette I, et al. Core outcome domains for lichen sclerosus: a CORALS initiative consensus statement. Br J Dermatol. 2023 May 1;188(5):628–35.

Pre-submission

Pre-submission

BSSVD
The British Society for the Study of Vulval Disease

15th November 2023

To whom it may concern,

Re: NIHR206718, PlatEleT-rich plasma and Autologous lipotransfer to treat vulvar fibrosis in Lichen Sclerosus – the PETALS multicentre randomised controlled feasibility trial

The BSSVD are fully supportive of this project and will provide input where needed to assist the progress of the study. The anticipated input is likely to include (but not limited to): contacting BSSVD members to complete survey work on the proposed trial design; focused feedback on specific clinical aspects of the study; engagement with potential future recruiting centres and dissemination of results.

Your sincerely

Mr David Nunns

Consultant Gynaeoncologist

BSSVD Chair

admin@bssvd.org

Pre-submission

11. Administrative contact details

Administrative contact name	Mike Dinan
Administrative contact job title	Director of Financial Operations at Royal Free London NHS
Administrative contact telephone number	020 7794 0500
Administrative contact email address	michael.dinan@nhs.net

12. Research & Development office contact

R&D contact name	Farhan Naim
R&D contact job title	Director of Research and Development at Royal Free London NHS Trust
R&D contact telephone number	020 7794 0500
R&D contact email address	farhan.naim@nhs.net

13. Acknowledgement, review and submit

COI declaration Please declare any conflicts or potential conflicts of interest that you or your co-applicants may have in undertaking this research, including any relevant personal, non-personal and commercial interest that could be perceived as a conflict of interest.
No conflict of interest to declare

Lead Applicant - Agreement to terms and conditions

In **ticking** this, you as **Lead Applicant** confirm that the information given on this form is correct and that you will be actively engaged in this research and responsible for its overall management. In addition, you will accept responsibility for ensuring that the host institution and interested parties are kept informed. **Ticking this box constitutes an electronic signature of the Lead Applicant with regard to this application**
☒Confirmed

Appropriate and relevant involvement of patients/service users, carers and the public
☒Confirmed

A good quality plain English summary	☒Confirmed

A clear description of team member roles and contribution
☒Confirmed

Pre-submission

A clear scientific abstract
☒Confirmed

A clear description of the changes from first stage
☒Confirmed

A flow diagram illustrating the study design / flow of participants (document upload), if appropriate
☒Confirmed

A full and accurate detailed budget breakdown
☒Confirmed

A clear justification of costs / value for money
☒Confirmed

References (document upload)
☒Confirmed

A clear Detailed Research Plan outlining the study design, methods, dissemination etc.
☒Confirmed

A CTU letter of support if required (document upload)
☒Confirmed

Completed Schedule of Events Cost Attribution Tool (SoECAT)
☒Confirmed

14. CV - Lead Applicant(s)

Full Name	Professor Peter Butler	**Position**	Director of The Charles Wolfson Centre for Reconstructive Surgery
Department	Department of Plasic Surgery, Division of Surgery and Interventional Science	**Institution**	Royal Free Hospital
ORCID iD	0000-0001-7419-1493	**Telephone No.**	02079083774
Address Line 1	Royal Free Hospial	**Country**	United Kingdom (England)
Address Line 2	Pond Street	**Postcode**	NW32QG
Address Line 3		**Web Page.**	

Qualification	Subject	Institution	From	To
MD	Medicine	University College Dublin	September 2001	September 2003
MD	MD	Royal College of Surgeons in Ireland	September 2000	May 2005
FRCS(Plast)	Plastics	Royal College of Surgeons	September 1994	May 1998

Pre-submission

FRCS(Plas)	Plastic Surgery	Intercollegiate Board of Royal Colleges	September 1992	September 1998
FRCS(Eng)	Surgery	Royal College of Surgeons in England	September 1990	September 1992
FRCSI	Surgery	Royal College of Surgeons in Ireland	September 1989	September 1991
BSc	-	University College Dublin	September 1989	May 1990
LRCPI LRCSI	-	Royal College of Surgeons in Ireland	September 1981	May 1987
MB BaO BCh	Medicine	University College Dublin	September 1981	June 1987

Selected recent relevant publication(s)

Publications
Almadori, AA., Speiser, SS., Ashby, IA., Lacher, RL., Bishop, DB., Mosahebi, A. and Butler, P.. (2022) 'Portable three-dimensional imaging to monitor small volume enhancement in face, vulva, and hand: A comparative study', *Journal of Plastic Reconstructive and Aesthetic Surgery*, 9(75).
Almadori, AA., Zenner, NZ., Boyle, DB., Swale, VS., Reid, WR., Maclean, AM. and Butler, P.. (2020) 'Development and Validation of a Clinical Grading Scale to Assess the Vulvar Region: The Vulvar Architecture Severity Scale', *Aesthetic surgery journal*, 12(40).
Almadori, AA., Hansen, EH., Boyle, DB., Zenner, NZ., Swale, VS., Reid, W., Maclane, AM. and Butler, P.. (2020) 'Fat Grafting Improves Fibrosis and Scarring in Vulvar Lichen Sclerosus: Results From a Prospective Cohort Study', *Journal of lower genital tract disease*, 3(24).
(2016) ' Naderi N, Combellack EJ, Griffin M, Sedaghati T, Javed M, Findlay MW, Wallace CG, Mosahebi A, Butler PE, Seifalian AM, Whitaker IS. The regenerative role of adipose-derived stem cells (ADSC) in plastic and reconstructive surgery. Int Wound J. 2017 Feb;14(1):112-124. ',
(2017) ' Griffin M, Ryan CM, Pathan O, Abraham D, Denton CP, Butler PE. Characteristics of human adipose derived stem cells in scleroderma in comparison to sex and age matched normal controls: implications for regenerative medicine. Stem Cell Res Ther. 2017 ',
(2016) ' Kumar R, Griffin M, Adigbli G, Kalavrezos N, Butler PE. Lipotransfer for radiation-induced skin fibrosis. Br J Surg. 2016 Jul;103(8):950-61. doi: 10.1002/bjs.10180. ',

Grants

Start Date	Duration	Amount	Source	Title	Role of Applicant
01/07/2022	24	3,440.00	Health Education England	Simulation Training Grant Microsurgical Skills Simulation Training	Primary Investigator
01/07/2021	36	12,000.00	British Society of the Study of Vulval Disease	An In Vitro Study of Adipose Derived Stem Cells and Platelet Enriched Plasma in Lichen Sclerosus	Primary Investigator
01/02/2019	24	245,985.00	CCF	Sys-Stem (Feasibility study of stem-cell enriched autologous lipotransfer to treat oro-	Lead Applicant

Pre-submission

Grants

Start Date	Duration	Amount	Source	Title	Role of Applicant
				facial fibrosis in systemic sclerosis)	
01/09/2017	36	15,000.00	Scleroderma & Raynauds UK (SRUK)	Mechanistic study of autologous lipotransfer in SSc. Analysis of lipoaspirate in the treatment of scleroderma	Lead
01/07/2016	72	3,000,000.00	The Charles Wolfson Foundation	Capital Funding Grant: The Centre of Reconstruction Surgery	Lead Applicant

15. CV - Co-applicants

Co Applicant - 1

Full Name Miss Aurora Almadori

Organisation Royal Free and University College Medical School

Email Address Aurora.almadori@nhs.net

Web Page.

Education

From	To	Qualification	Subject	Country	Organisation	Department
03/2017	03/2023	PhD	Regenerative Surgery	United Kingdom	University College London	Nanotechnology
09/2020	09/2021	Diploma	Pelvi-Perineal Medicine and Reconstructive Plastic Surgery	France	Universitè de Montpellier	Gynaecology
09/2012	07/2017	Specialisation Degree in Plastic Surgery	Plastic, Reconstuctive, and Aesthetic Surgery	Italy	University of Campania	Surgery
09/2014	09/2015	MSc	Burns, Plastic and Reconstructive Surgery	United Kingdom	University College London	Nanotechnology
09/2013	09/2014	MSc	Migration and Migrant Health Care	Italy	Catholic University of the Sacred Heart	Medicine
09/2004	03/2011	MBBS(Hons)	Medicine and Surgery	Italy	Catholic University of the Sacred Heart	Surgery

Publications

Almadori A, Hansen E, Boyle D, Zenner N, Swale V, Reid W, Maclane A and Butler PEM. (2020) 'Fat Grafting Improves Fibrosis and Scarring in Vulvar Lichen Sclerosus: Results From a Prospective Cohort Study.', *Journal of lower genital tract disease*.

Pre-submission

Publications

Griffin MF, Drago J, Almadori A, Kalavrezos N and Butler PE. (2019) 'Evaluation of the efficacy of lipotransfer to manage radiation-induced fibrosis and volume defects in head and neck oncology.', *Head & Neck*.
Serra-Mestre JM, Serra-Renom JM, Martinez L, Almadori A and D'Andrea F. (2014) 'Platelet-rich plasma mixed-fat grafting: a reasonable prosurvival strategy for fat grafts?', *Aesthetic plastic surgery*.
Aurora Almadori, Michelle Griffin, Caroline M. Ryan, Debbie F. Hunt, Esther Hansen, Ravi Kumar, David J. Abraham, Christopher P. Denton and Peter E. M. Butler. 'Stem cell enriched lipotransfer reverses the effects of fibrosis in systemic sclerosis', *PLoS ONE*.
Aurora Almadori, Nicole Zenner, Deborah Boyle, Victoria Swale, Wendy Reid, Allan Maclean and Peter E M Butler. 'Development and Validation of a Clinical Grading Scale to Assess the Vulvar Region: The Vulvar Architecture Severity Scale', *Aesthetic surgery journal*.
James I, Bourne D, Silva M, Havis E, Albright K, Zhang L, Kostereva N, Wang S, DiBernardo G, Guest R, Lei J, Almadori A, Satish L, Marra K and Rubin JP. (2018) 'Adipose stem cells enhance excisional wound healing in a porcine model.', *The Journal of surgical research*.

Grants

Start Date	Duration	Amount	Source	Title	Role of Applicant
01/01/2024	24	2,700.00	SICPRE - Italian Society of Plastic, Reconstructive-Regenerative, and Aesthetic Surgery	Under 40 Bursary	Main applicant
01/07/2019	48	10,100.00	BSSVD - British Society for the Study of Vulvo-vaginal Disease	Research Award	Main Applicant
01/12/2017	36	36,000.00	IMSOGB - Italian medical Society of Great Britain	Regenerative Medicine Fellowship	Main Applicant

Co Applicant - 2

Full Name	Dr Hakim-Moulay Dehbi	**Organisation**	University College London
Email Address	h.dehbi@ucl.ac.uk	**Web Page.**	

Education

From	To	Qualification	Subject	Country	Organisation	Department
09/2013	09/2016	PhD	Biostatistics		Imperial College London	Epidemiology and biostatistics

Publications

Dehbi, HM., O'Quigley, J. and Iasonos, A.. (2022) 'Controlled amplification in oncology dose-finding trials.', *Contemporary Clinical Trials*, 125.
Dehbi, HM., O'Quigley, J. and Iasonos, A.. (2021) 'Controlled backfill in oncology dose-finding trials.', *Contemporary Clinical Trials*, 111.
Dehbi, HM., Lowe, DM. and O'Quigley, J.. (2020) 'Early phase dose-finding trials in virology.', *Statistics in medicine*, 40(2).
Dehbi, HM. and Hackshaw, A.. (2020) 'Sample size calculation in randomised phase II selection trials using a margin of practical equivalence.', *Trials*, 21(1).
Dehbi, HM., Mallick, U., Wadsley, J., Newbold, K., Harmer, C. and Hackshaw, A.. (2018) 'Recurrence after low-dose radioiodine ablation and recombinant human thyroid-stimulating hormone for differentiated thyroid cancer (HiLo): long-term results of an open-label, non-inferiority randomised controlled trial.', *The lancet. Diabetes & endocrinology*,

Pre-submission

Publications

7(1).

Grants

Start Date	Duration	Amount	Source	Title	Role of Applicant
01/12/2022	30	246,866.00	CCF	A feasibility multicentre randomised controlled trial comparing Wide-awake Local Anaesthesia No Tourniquet (WALANT) versus General and Regional Anaesthesia for Flexor Tendon Repair (WAFER Trial)	Co Applicant
01/09/2022	24	249,705.00	CCF	A MULTI-centre feasibility study to assess the use of 99m Tc-SestaMIBI SPECT/CT in the diagnosis of kidney tumours (MULTI-MIBI study).	Co Applicant
01/11/2021	36	1,900,000.00	NIHR UKRI	Nicotinamide in Glaucoma (NAMinG): a randomised, placebo-controlled, multicentre, Phase III trial	co-applicant

Co Applicant - 3

Full Name Dr Esther Hansen **Organisation** Royal Free Hospital
Email Address esther.hansen@nhs.net **Web Page.**

Education

From	To	Qualification	Subject	Country	Organisation	Department
09/2011	04/2015	Intermediate Level Diploma	Intermediate Level Diploma in Systemic Psychotherapy	United Kingdom	Association of Family Therapy	Family Therapy
09/2006	07/2007	Foundation Level Diploma	Foundation Level Diploma in Systemic Psychotherapy	United Kingdom	Assocation of Family Therapy	Family Therapy
09/2001	09/2004	DClinPsy	Professional Doctorate in Clinical Psychology	United Kingdom	University of East London	School of Psychology
09/2000	07/2001	Postgraduate Diploma	Counselling Skills	United Kingdom	University of North London	Psychology
09/1996	07/1999	BSc	Psychology	United Kingdom	University of Birmingham	School of Psychology
09/1989	07/1996	Diplome de Fin d'Etudes Secondaires	Diplome de Fin d'Etudes Secondaires		Ministere de L'Education Nationale	Ecole Privee Fieldgen

Pre-submission

Publications

Norbert Venantius Kang, Alexander Woollard, Dominika Antonina Michno, Yazan Al-Ajam, Jaclyn Tan and Esther Hansen. (2022) 'A consecutive series of targeted muscle reinnervation (TMR) cases for relief of neuroma and phantom limb pain: UK perspective', *Journal of Plastic, Reconstructive & Aesthetic Surgery*.
E. Guest, N. Paraskeva, C. Griffiths, E. Hansen, A. Clarke, E. Baker and D. Harcourt. (2021) 'The nature and importance of women's goals for immediate and delayed breast reconstruction', *Journal of Plastic, Reconstructive & Aesthetic Surgery*.
Alex Clarke, N. Paraskeva, P. White, P. Tollow, E. Hansen and D. Harcourt. (2021) 'PEGASUS: the Design of an Intervention to Facilitate Shared Decision-making in Breast Reconstruction', *Journal of Cancer Education*.
Aurora Almadori, Esther Hansen, Deborah Boyle, Nicole Zenner, Victoria Swale, Wendy Reid, Allan Maclane and Peter E.M. Butler. (2020) 'Fat Grafting Improves Fibrosis and Scarring in Vulvar Lichen Sclerosus: Results From a Prospective Cohort Study', *Journal of lower genital tract disease*.
J. Joseph, P. Randhawa, S.A. Hannan, J. Long, S. Goh, N. O'Shea, H. Saleh, E. Hansen, D. Veale and P. Andrews. (2017) 'Body dysmorphic disorder in patients undergoing septorhinoplasty surgery: should we be performing routine screening?', *Clinical Otolaryngology*.
Aurora Almadori, Michelle Griffin, Caroline M. Ryan, Debbie F. Hunt, Esther Hansen, Ravi Kumar, David J. Abraham, Christopher P. Denton and Peter E. M. Butler. 'Stem cell enriched lipotransfer reverses the effects of fibrosis in systemic sclerosis', *PLoS ONE*.

Grants

Start Date	Duration	Amount	Source	Title	Role of Applicant
01/02/2019	24	245,985.00	CCF	Sys-Stem (Feasibility study of stem-cell enriched autologous lipotransfer to treat oro-facial fibrosis in systemic sclerosis)	Co Applicant

Co Applicant - 4

Full Name Dr Sara Jasionowska **Organisation** Royal Free Hospital
Email Address s.jasionowska@nhs.net **Web Page.** https://www.charleswolfsoncentre.com/

Education

From	To	Qualification	Subject	Country	Organisation	Department
04/2021	06/2023	MRCS fellowship of Royal College of Surgeons	MRCS fellowship	United Kingdom	Royal College of Surgeons of England	Surgery
05/2021	07/2021	Fellow of the Higher Education Academy (D2)	Education	United Kingdom	Imperial College of Science, Technology and Medicine	Education
09/2014	07/2020	MBBS(Hons)	Medicine	United Kingdom	Queen Mary University of London	Medicine
09/2017	07/2018	BSc	Anatomy, Developmental and Human Biology	United Kingdom	King's College London	Anatomy

Publications

Pre-submission

Publications

Frommer ML, Benjamin Langridge, Awad L, Jasionowska S, Denton CP, Abraham DJ, Abu-Hanna J and Butler PEM. (2023) 'Single-Cell Analysis of ADSC Interactions with Fibroblasts and Endothelial Cells in Scleroderma Skin.', *Cells*.
Langridge BJ, Jasionowska S, Khan H, Awad L, Turner BRH, Varghese J and Butler PEM. 'Achieving optimal clinical outcomes in autologous fat grafting: A systematic review of processing techniques.', *Journal of plastic, reconstructive & aesthetic surgery : JPRAS*.
Benedict R. H. Turner, Machin M, Jasionowska S, Salim S, Onida S, Shalhoub J and Davies AH. (2022) 'Systematic Review and Meta-analysis of the Additional Benefit of Pharmacological Thromboprophylaxis for Endovenous Varicose Vein Interventions.', *Annals of surgery*.
Benedict R. H. Turner, Jasionowska S, Machin M, Javed A, Adam Gwozdz, Shalhoub J, Onida S and Davies AH. (2022) 'Systematic review and meta-analysis of exercise therapy for venous leg ulcer healing and recurrence.', *Journal of vascular surgery. Venous and lymphatic disorders*.
EuroSurg Collaborative. (2019) 'Safety and efficacy of non-steroidal anti-inflammatory drugs to reduce ileus after colorectal surgery.', *The British journal of surgery*.
Turner BRH, Machin M, Salih M, Jasionowska S, Lawton R, Francesca Siracusa, Gwozdz AM, Shalhoub J and Davies AH. 'An Updated Systematic Review and Meta-Analysis of the Impact of Graduated Compression Stockings in Addition to Pharmacological Thromboprophylaxis for Prevention of Venous Thromboembolism in Surgical Inpatients.', *Annals of surgery*.

Grants

Start Date	Duration	Amount	Source	Title	Role of Applicant
01/04/2023	39	2,673,754.00	NIHR HTA	Inpatient GRAduated Compression stocking use as an adjunct to Extended duration pharmacoprophylaxis for venous thromboembolism prevention – the GRACE multi-centre randomised controlled trial	Co-applicant FTE 5%
01/01/2023	6	5,000.00	Malcolm Coptcoat Foundation	Malcolm Coptcoat Undergraduate Research Grant for Development and content validation of the urethroplasty training and assessment tool (UTAT) for the dorsal onlay BMG urethroplasty.	Main applicant
01/01/2022	6	500.00	NIHR BRC	Pharmacomechanical and mechanical thrombectomy for acute iliofemoral DVT in the prevention of post-thrombotic syndrome and long-term quality of life outcomes - PPI before setting up a pilot study	Main applicant

Co Applicant - 5

Full Name	Dr Benjamin Langridge	**Organisation**	Royal Free Hospital
Email Address	Zchabla@ucl.ac.uk	**Web Page.**	

Education

From	To	Qualification	Subject	Country	Organisation	Department

Pre-submission

Education

From	To	Qualification	Subject	Country	Organisation	Department
10/2019	07/2023	MRCS	Membership of the Royal College of Surgeons	United Kingdom	Royal College of Surgeons	Royal College of Surgeons
07/2018	07/2023	FHEA	Fellowship of the Higher Education Academy	United Kingdom	Higher Education Academy	Higher Education Academy
09/2019	08/2021	MSc(Hons)	Burns, Plastic and Reconstructive Surgery	United Kingdom	University College London	division of surgery
10/2011	07/2017	MBBS(Hons)	Medicine	United Kingdom	University College London	Medicine
09/2011	07/2017	MA	MA - Physiology, Development and Neuroscience.	United Kingdom	University of Cambridge	Gonville and Caius College
10/2011	09/2014	BA(Hons)	Physiology, Development and Neuroscience. Gonville and Caius College, University of Cambridge	United Kingdom	Gonville and Caius College, University of Cambridge	Medicine

Publications

Marvin L. Frommer, Benjamin J. Langridge, Laura Awad, Sara Jasionowska, Christopher P. Denton, David J. Abraham, Jeries Abu-Hanna and Peter E. M. Butler. (2023) 'Single-Cell Analysis of ADSC Interactions with Fibroblasts and Endothelial Cells in Scleroderma Skin', *Cells*.
(2023) 'Langridge BJ, Jasionowska S, Khan H, Awad L, Turner BRH, Varghese J, Butler PEM. Achieving optimal clinical outcomes in autologous fat grafting: A systematic review of processing techniques. J Plast Reconstr Aesthet Surg. 2023 Jun;81:9-25. doi: 10.1016/j.bjps.2023.01.003. Epub 2023 Feb 13. PMID: 37075610.',
(2021) 'Langridge BJ, Jeon FHK, Almadori A, Griffin MF, Denton CP, Butler PEM. Autologous lipotransfer can improve the outcomes of localised scleroderma. Clin Exp Rheumatol. 2021 Jul-Aug;39 Suppl 131(4):159. doi: 10.55563/clinexprheumatol/uczb6s. Epub 2021 Mar 26. PMID: 33822708.',
(2020) 'Jeon FHK, Langridge B, Ruston J, Griffin MF, Butler PEM. Autologous fat grafting for plantar fat-pad atrophy in systemic sclerosis. Clin Exp Rheumatol. 2020 May-Jun;38 Suppl 125(3):180. Epub 2020 Apr 22. PMID: 32324119.',
Kenworthy W, Langridge B, Patel N and Waterhouse N. (2016) 'Use of Platelet Preparations in Facial Rejuvenation and Wound Healing Remains Unproven.', *Aesthetic plastic surgery*.
(2021) 'Speiser S, Langridge B, Birkl MM, Kubiena H, Rodgers W. Update on Noma: systematic review on classification, outcomes and follow-up of patients undergoing reconstructive surgery after Noma disease. BMJ Open. 2021 Aug 5;11(8):e046303. doi: 10.1136/bmjopen-2020-046303. PMID: 34353795; PMCID: PMC8344268.',

Grants

Start Date	Duration	Amount	Source	Title	Role of Applicant
01/05/2023	12	500.00	Association of Surgeons in Training	ASIT Surgical Training and Research Grant	co-applicant
01/04/2023	12	5,000.00	British Society for Surgery of the Hand	BSSH Educational Grant	co-applicant
01/08/2022	12	2,900.00	Faculty of Surgical Trainers & Association for the Study of Medical Education	FST/ASME Educational Research Grant	co-applicant

Pre-submission

Co Applicant - 6

Full Name	Miss Monica Panca	**Organisation**	University College London
Email Address	m.panca@ucl.ac.uk	**Web Page.**	

Education

From	To	Qualification	Subject	Country	Organisation	Department
09/2007	02/2009	MSc	Economic Evaluation in Health Care	United Kingdom (England)	City University, London	Economics
10/2000	07/2004	BSc	Economics, Major in Foreign Trade		Tibiscus University, Timisoara, Romania	Economic Science

Publications

Panca, M., Blackstone, J., Stirrup, O., Cutino-Moguel, MT., Thomson, E., Peters, C., Snell, LB., Nebbia, G., Holmes, A., Chawla, A., Machin, N., Taha, Y., Mahungu, T., Saluja, T., de Silva, TI., Saeed, K., Pope, C., Shin, GY., Williams, R., Darby, A., Smith, DL., Loose, M., Robson, SC., Laing, K., Partridge, DG., Price, JR. and Breuer, J.. (2023) 'Evaluating the cost implications of integrating SARS-CoV-2 genome sequencing for infection prevention and control investigation of nosocomial transmission within hospitals.', *The Journal of hospital infection*, 139.

Sampson, EL., Feast, A., Blighe, A., Froggatt, K., Hunter, R., Marston, L., McCormack, B., Nurock, S., Panca, M., Powell, C., Rait, G., Robinson, L., Woodward-Carlton, B., Young, J. and Downs, M.. (2020) 'Pilot cluster randomised trial of an evidence-based intervention to reduce avoidable hospital admissions in nursing home residents (Better Health in Residents of Care Homes with Nursing-BHiRCH-NH Study).', *BMJ Open*, 10(12).

Farris, O., Royston, R., Absoud, M., Ambler, G., Barnes, J., Hunter, R., Kyriakopoulos, M., Oulton, K., Paliokosta, E., Panca, M., Paulauskaite, L., Poppe, M., Ricciardi, F., Sharma, A., Slonims, V., Summerson, U., Sutcliffe, A., Thomas, M. and Hassiotis, A.. (2020) 'Clinical and cost effectiveness of a parent mediated intervention to reduce challenging behaviour in pre-schoolers with moderate to severe intellectual disability (EPICC-ID) study protocol: a multi-centre, parallel-group randomised controlled trial.', *BMC Psychiatry*, 20(1).

Livingston, G., Barber, J., Marston, L., Stringer, A., Panca, M., Hunter, R., Cooper, C., Laybourne, A., La Frenais, F., Reeves, S., Manela, M., Lambe, K., Banerjee, S. and Rapaport, P.. (2019) 'Clinical and cost-effectiveness of the Managing Agitation and Raising Quality of Life (MARQUE) intervention for agitation in people with dementia in care homes: a single-blind, cluster-randomised controlled trial.', *The lancet. Psychiatry*, 6(4).

Panca, M., Livingston, G., Barber, J., Cooper, C., La Frenais, F., Marston, L., Cousins, S. and Hunter, RM.. (2019) 'Healthcare resource utilisation and costs of agitation in people with dementia living in care homes in England - The Managing Agitation and Raising QUality of LifE in Dementia (MARQUE) study.', *PLoS ONE*, 14(2).

Panca, M., Buszewicz, M., Strydom, A., Hassiotis, A., Welch, CA. and Hunter, RM.. (2018) 'Resource use and cost of annual health checks in primary care for people with intellectual disabilities.', *Journal of intellectual disability research : JIDR*, 63(3).

Grants

Start Date	Duration	Amount	Source	Title	Role of Applicant
01/07/2023	48	1,861,407.00	Health Technology Assessment (HTA)	Antidepressants to prevent relapse in depression in older people (ANTLER 75+ Trial)	Co-applicant
01/03/2023	72	1,893,811.00	NIHR HTA	Developmental Optimisation using Lavage in Preterm intraventricular Haemorrhage in Neonates (ENLIVEN-UK)	Co-applicant

Pre-submission

Grants

Start Date	Duration	Amount	Source	Title	Role of Applicant
01/12/2022	30	246,866.00	CCF	A feasibility multicentre randomised controlled trial comparing Wide-awake Local Anaesthesia No Tourniquet (WALANT) versus General and Regional Anaesthesia for Flexor Tendon Repair (WAFER Trial)	Co Applicant
01/01/2022	48	154,287.00	NIHR	Randomised Evaluation of Sodium Dialysate Levels on Vascular Events (RESOLVE)	Co-applicant

Co Applicant - 7

Full Name Dr Rosalind Simpson **Organisation** University of Nottingham
Email Address rosalind.simpson@nottingham.ac.uk **Web Page.**

Education

From	To	Qualification	Subject	Country	Organisation	Department
07/2018	08/2018	Certificate fo Completion of Training (CCT)	Dermatology	United Kingdom	Royal College of Physicians	Dermatology
09/2011	08/2015	PhD	Dermatology	United Kingdom	University of Nottingham	School of Medicine
08/2014	08/2015	PGCert	Evidence Based Health Care	United Kingdom	University of Oxford	Continuing Medical Education
09/2010	10/2010	Speciality Certificate Examination (SCE)	Dermatology	United Kingdom	Royal College of Physicians	Dermatology
09/1999	08/2004	MBBS	Medicine/surgery	United Kingdom (England)	University of Nottingham	Medical School
08/2002	12/2002	BMedSci	Public Health and Epidemiology	United Kingdom (England)	University of Nottingham	Medical School, Public Health and Epidemiology

Publications

R.C. Simpson, S.M. Cooper, G. Kirtschig, S. Larsen, S. Lawton, M. McPhee, R. Murphy, D. Nunns, S. Rees, M. Tarpey and K.S. Thomas. (2019) 'Future research priorities for lichen sclerosus – results of a James Lind Alliance Priority Setting Partnership', *British Journal of Dermatology*.
Rosalind C. Simpson, Ruth Murphy, Daniel J. Bratton, Matthew R. Sydes, Sally Wilkes, Helen Nankervis, Shelley Dowey, Hazel Bell, Margaret Cruickshank, Karen Gibbon, Cathy M. Green, Christina Wong, Caroline M. Owen, Kate London, Shaheen Haque and Kim S. Thomas. (2018) 'Help for Future Research: Lessons Learned in Trial Design, Recruitment, and Delivery From the "hELP" Study', *Journal of lower genital tract disease*.
Rosalind C Simpson, Gudula Kirtschig, Amanda Selk, Suzanne von Seitzberg, Gitte Vittrup, Ione Bissonnette, Jan Kottner, Jaclyn Lanthier, Chris Stanton, David Foster, Martin Promm, Angelo Augenti, Stefano Lauretti, Kim Thomas and the Core Outcomes for Research in Lichen Sclerosus (CORALS) initiative steering group. 'Core outcome

Pre-submission

Publications

domains for lichen sclerosus: a CORALS initiative consensus statement', *British Journal of Dermatology*.
L. Kirby, S. Gran, F. Orekoya, C. Owen and R. Simpson. (2021) 'Is urinary incontinence associated with vulval lichen sclerosus in women? A cross-sectional study', *British Journal of Dermatology*.
F. Tasker, L. Kirby, D. J. C. Grindlay, F. Lewis and R. C. Simpson. (2021) 'Laser therapy for genital lichen sclerosus: A systematic review of the current evidence base', *Skin Health and Disease*.
S. Rees, L. Kirby and R.C. Simpson. 'Living with vulval lichen sclerosus: a systematic review', *British Journal of Dermatology*.

Grants

Start Date	Duration	Amount	Source	Title	Role of Applicant
01/03/2023	57	1,636,000.00	NIHR HTA (NIHR135121)	Proactive against reactive therapy for the prevention of lichen sclerosus exacerbation and progression of disease (PEARLS)	Lead Applicant
01/10/2022	24	9,600.00	UK Dermatology Clinical Trials Network	A Feasibility Study In The Form Of Focus Groups To Explore The Experiences And Real-life Outcomes For Patients With Anogenital Psoriasis	Lead Supervisor for Dermatology Trainee
01/09/2021	60	850,000.00	NIHR Advanced Fellowship (NIHR301434)	Addressing a neglected area of women's health: developing diagnostic criteria for vulval lichen sclerosus.	Lead Applicant
01/08/2019	12	10,000.00	British Skin Foundation (010_BSFBAD_19)	Is Urinary Incontinence associated with Lichen Sclerosus in Females?	Lead Supervisor for Dermatology Trainee
01/12/2018	24	10,000.00	UK Dermatology Clinical Trials Network	Developing a Core Outcome set for Lichen Sclerosus.	Lead Applicant.
01/01/2017	24	33,672.00	British Society for the Study of Vulval Disease Research Award.	Lichen Sclerosus Priority Setting Partnership	Lead Applicant
01/10/2012	36	330,000.00	NIHR Doctoral Research Fellowship	Vulval Erosive Lichen Planus	Lead Applicant
01/11/2010	12	9,600.00	NOTTINGHAM UNIVERSITY HOSPITALS CHARITY	Feasibility study for a multi-centre clinical trial studying treatment for vulval erosive lichen planus.	Co-applicant; designed, coordinated and collated r

Pre-submission

Co Applicant - 8

Full Name	Dr Victoria Swale	**Organisation**	Royal Free London NHS Foundation Trust
Email Address	victoria.swale@nhs.net	**Web Page.**	

Education

From	To	Qualification	Subject	Country	Organisation	Department
04/1999	04/2004	MD	Cancer genetics	United Kingdom	Queen Mary University of London	Medicine
02/2002	04/2004	FRCP	Medicine		Royal College of Physicians	N/A
07/1987	07/1992	Bachelor of Medicine	medicine	United Kingdom	University of Southampton	medicine

Publications

Almadori, A., Hansen, E., Boyle, D., Zenner, N., Swale, V., Reid, W., Maclane, A. and Butler, PEM.. (2020) 'Fat Grafting Improves Fibrosis and Scarring in Vulvar Lichen Sclerosus: Results From a Prospective Cohort Study.', *Journal of lower genital tract disease*, 24(3).
Nasir, S., Murray, C., Murray, C., Swale, VJ., Calonje, E. and Mazzon, S.. (2015) 'Indurated plaques on the penis.', *Clinical and Experimental Dermatology*, 41(1).
Rice, SA., Swale, VJ., Haque, R. and Rustin, MH.. (2013) 'Erythroderma in the emergency department.', *BMJ (Clinical research ed.)*, 346.
(2010) 'Cutaneous changes in familial cancer.',
Goulding, JM., Levine, S., Blizard, RA., Deroide, F. and Swale, VJ.. (2009) 'Dermatological surgery: a comparison of activity and outcomes in primary and secondary care.', *The British journal of dermatology*, 161(1).
Harwood, CA., McGregor, JM., Swale, VJ., Proby, CM., Leigh, IM., Newton, R., Khorshid, SM. and Cerio, R.. (2003) 'High frequency and diversity of cutaneous appendageal tumors in organ transplant recipients.', *Journal of the American Academy of Dermatology*, 48(3).

Grants

Start Date	Duration	Amount	Source	Title	Role of Applicant

Co Applicant - 9

Full Name	Mrs Maria Tether-Jenkins	**Organisation**	PPI representative
Email Address	maria_tether@hotmail.com	**Web Page.**	

Education

From	To	Qualification	Subject	Country	Organisation	Department

Publications

Grants

Start Date	Duration	Amount	Source	Title	Role of Applicant

Co Applicant - 10

Full Name	Professor Norman Williams	**Organisation**	University College London
Email Address	norman.williams@ucl.ac.uk	**Web Page.**	https://profiles.ucl.ac.uk/6692-norman-williams

Education

From	To	Qualification	Subject	Country	Organisation	Department

Pre-submission

Education

From	To	Qualification	Subject	Country	Organisation	Department
04/2014	11/2023	Fellow of the Institute of Biomedical Science	FIBMS	United Kingdom	Institute of Biomedical Science	Fellow
04/2012	11/2023	Fellow of the Royal Statistical Society	Fellow of the Royal Statistical Society	United Kingdom	Royal Statistical Society	Royal Statistical Society
01/2009	11/2023	CSci (Chartered Scientist)	Clinical Research	United Kingdom (England)	The Science Council	Institute of Clinical Research
10/1982	11/1992	PhD	Zinc and human pregnancy	United Kingdom (England)	Royal Postgraduate Medical School, University of London	Institute of Obstetrics and Gynaecology
10/1979	06/1981	BSc	Biochemistry and Microbiology	United Kingdom (England)	Royal Holloway College, University of London	Biochemistry, Microbiology

Publications

(2023) 'For a complete list of publications please see https://profiles.ucl.ac.uk/6692-norman-williams/publications',
Williams, NR.. (2023) 'Analysis of Clinical Trials and Review of Recent Advances in Therapy Decisions for Locally Advanced Prostate Cancer.', *Journal of Personalized Medicine*, 13(6).
Catto, JWF., Khetrapal, P., Ricciardi, F., Ambler, G., Williams, NR., Al-Hammouri, T., Khan, MS., Thurairaja, R., Nair, R., Feber, A., Dixon, S., Nathan, S., Briggs, T., Sridhar, A., Ahmad, I., Bhatt, J., Charlesworth, P., Blick, C., Cumberbatch, MG., Hussain, SA., Kotwal, S., Koupparis, A., McGrath, J., Noon, AP., Rowe, E., Vasdev, N., Hanchanale, V., Hagan, D., Brew-Graves, C. and Kelly, JD.. (2022) 'Effect of Robot-Assisted Radical Cystectomy With Intracorporeal Urinary Diversion vs Open Radical Cystectomy on 90-Day Morbidity and Mortality Among Patients With Bladder Cancer: A Randomized Clinical Trial.', *JAMA*, 327(21).
Treasure, T., Farewell, V., Macbeth, F., Monson, K., Williams, NR., Brew-Graves, C., Lees, B., Grigg, O. and Fallowfield, L.. (2019) 'Pulmonary Metastasectomy versus Continued Active Monitoring in Colorectal Cancer (PulMiCC): a multicentre randomised clinical trial.', *Trials*, 20(1).
Kasivisvanathan, V., Rannikko, AS., Borghi, M., Panebianco, V., Mynderse, LA., Vaarala, MH., Briganti, A., Budäus, L., Hellawell, G., Hindley, RG., Roobol, MJ., Eggener, S., Ghei, M., Villers, A., Bladou, F., Villeirs, GM., Virdi, J., Boxler, S., Robert, G., Singh, PB., Venderink, W., Hadaschik, BA., Ruffion, A., Hu, JC., Margolis, D., Crouzet, S., Klotz, L., Taneja, SS., Pinto, P., Gill, I., Allen, C., Giganti, F., Freeman, A., Morris, S., Punwani, S., Williams, NR., Brew-Graves, C., Deeks, J., Takwoingi, Y., Emberton, M. and Moore, CM.. (2018) 'MRI-Targeted or Standard Biopsy for Prostate-Cancer Diagnosis.', *The New England journal of medicine*, 378(19).
Fung, V., Kohlhardt, S., Vergani, P., Zardin, GJ. and Williams, NR.. (2017) 'Intraoperative prediction of the two axillary lymph node macrometastases threshold in patients with breast cancer using a one-step nucleic acid cytokeratin-19 amplification assay.', *Molecular and Clinical Oncology*, 7(5).

Grants

Start Date	Duration	Amount	Source	Title	Role of Applicant
01/12/2022	30	246,866.00	CCF	A feasibility multicentre randomised controlled trial comparing Wide-awake Local Anaesthesia No Tourniquet (WALANT) versus General and Regional Anaesthesia for	Co Applicant

Pre-submission

Grants

Start Date	Duration	Amount	Source	Title	Role of Applicant
				Flexor Tendon Repair (WAFER Trial)	
02/08/2021	36	249,918.00	CCF	Videoendoscopic radical inguinal lymphadenectomy versus open radical inguinal lymphadenectomy in patients with penile cancer: a feasibility randomised controlled trial	Co Applicant
13/05/2019	36	244,271.00	CCF	A feasibility study of a cohort embedded randomised control trial comparing NEphron Sparing Treatment (NEST) for small renal masses.	Co Applicant
01/04/2019	58	249,839.00	CCF	A triple-blind randomised placebo-controlled feasibility study to assess the impact of octreotide infusion during liver transplantation on post-operative renal failure.	Co Applicant
01/04/2018	60	232,535.00	CCF	Improving outcomes for men undergoing surgery for prostate cancer. A study to evaluate the feasibility of a multicentre randomised controlled trial of frozen section technology to improve oncological and functional outcomes at robotic radical prostatectomy.	Co Applicant
01/01/2018	75	1,444,811.00	CCF	A multi-modality, surgical planning and guidance system to improve the up-take of laparoscopic liver resection	Co Applicant

Pre-submission